



Situation Analysis

MYANMAR CRISIS

Period: 01/06/26 – 07/06/26

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Introduction

Purpose

This report provides a comprehensive overview of the current humanitarian situation, focusing on both operational environment factors and sectoral analysis. It aims to support informed decision-making for humanitarian partners and stakeholders.

About GANNET

GANNET is a Generative AI Initiative from Data Friendly Space, offering AI-powered tools built to make knowledge and real-time information accessible to our community in a resource-efficient and ethical way. It provides tools such as the GANNET Virtual Assistant and GANNET SituationHub to help humanitarian actors access, analyze, and utilize critical information for their operations.

GANNET SituationHub

The GANNET SituationHub is a generative AI-powered digital public good designed to identify and analyze humanitarian data from thousands of trusted sources in real time. By transforming complex data into clear insights, it enables faster, more effective response strategies and improved coordination among stakeholders.

GANNET Virtual Assistant

The GANNET Virtual Assistant is a verified chatbot designed for humanitarian quick decision-making. Built on cutting-edge Generative AI and trained with humanitarian expert-reviewed data, it provides real-time information. The GANNET Virtual Assistant facilitates rapid data access and synthesis for crisis response.

Methodology Overview

The Situation Hub's analytical process uses a structured Human-in-the-Loop workflow that combines automated analysis retrieval through GANNET Virtual Assistant, DFS's AI-powered humanitarian assistant, with expert review by the DFS analysis team. GANNET automates the extraction and generation of crisis-related insights based on structured prompts, which are aligned with a tailored analytical framework. These outputs are then reviewed, validated, and refined by analysts to ensure accuracy, contextual relevance, and operational utility. The workflow is organized into four key phases: planning and setup, ongoing operations, quality assurance, and system evolution, ensuring a balance between speed and analytical rigor.

Planning, Setup & Quality Control

In the planning phase, analysts develop a contextualized analytical framework, largely based on the Joint Intersectoral Analysis Framework (JIAF), and convert it into structured prompts for AI-assisted data extraction. These prompts are refined over time to improve output quality. Data sources are vetted for reliability, and translations are reviewed for accuracy. Using a centralized Content Management System (CMS), analysts review and curate AI-generated outputs, validate them against external sources, and make refinements to enhance clarity and relevance. The CMS enables collaborative editing and version tracking, ensuring outputs align with humanitarian needs.

Bias Mitigation

To minimize bias, the team diversifies data sources, cross-verifies information, and continuously evaluates the AI's performance. Analysts are crucial in identifying and correcting inaccuracies, especially in automated categorization. Future improvements aim to enhance source credibility checks and sentiment analysis, further strengthening the reliability of the Situation Hub's outputs.

Crisis Overview

The SituationHub landing pages use a standardized yet customizable template to deliver consistent, comprehensive crisis overviews. These pages integrate static content with dynamic data from sources like ACLED and DTM via open APIs, enabling interactive visuals with real-time updates. Regular reviews and verification with in-country partners ensure accuracy, while manual updates are used for sources not connected to APIs.

Media Monitoring

The media monitoring process has evolved from a manual, list-based approach to an AI-assisted system integrated into the CMS. This allows analysts to efficiently review, filter, and contextualize media narratives, while automation helps track emerging trends.

Overview

The following overview has been generated using the information available up to June 29, 2026. It provides a synthesized summary and key insights into the crisis based on the most recent data accessible at that time.

Summary

Myanmar has been in crisis since 1 February 2021, when the military (Tatmadaw) overthrew the democratically elected government. This coup sparked widespread protests and armed resistance, leading to a severe political, economic, and humanitarian crisis. The military regime, known as the State Administration Council (SAC), has responded with brutal force, resulting in thousands of civilian deaths and widespread human rights abuses. As of March 2026, approximately 3.7 million people are internally displaced across the country—a dramatic increase from roughly 300,000 pre-2021—with cross-border displacement projected to rise toward 5.2 million in 2026.

In attempt to seek international legitimacy, the military held general elections in three phases over December 2025 and January 2026 for the first time since the coup. The overwhelming majority of seats were won by the pro-military Union Solidarity and Development Party (USDP) and saw SAC chairman Min Aung Hlaing elected as president in April 2026. The elections have been widely denounced as neither free nor fair. Despite the transition to a quasi-civilian presidency, armed clashes have continued in multiple parts of the country, including Sagaing, Magway, northern Shan, Kayah, Kayin, Kachin, Chin, and Rakhine. The Arakan Army (AA) now controls all but three of Rakhine State's 17 townships, while resistance forces including the Karen National Liberation Army (KNLA) have consolidated influence across parts of central and southeastern Myanmar. The UN's 2026 Humanitarian Needs and Response Plan (HNRP), released 10 December 2025, projects over 16 million people will require humanitarian assistance this year.

Recovery from the March 2025 earthquake remains slow, with an estimated 3.5 million tonnes of debris and reconstruction hampered by insecurity, shortages of heavy machinery (43%) and labour (40%), and limited access. Many affected communities, particularly in Magway, Sagaing, and Mandalay, continue to live in temporary shelters, while essential services remain disrupted. Approximately 24,200 houses were damaged (24% completely destroyed), and 132 health facilities were damaged—half in Mandalay.

The Rohingya crisis continues to worsen, with the junta's blockade of Rakhine preventing international aid delivery since late 2023, leaving hundreds of thousands of IDPs with severely limited assistance. On 10 December 2025, a military airstrike destroyed Mrauk-U General Hospital, killing at least 33 people and injuring 76–80—one of the deadliest attacks on healthcare in the conflict. Chronic poverty now affects over 80% of the population; the World Bank projects GDP will contract 2.0% in FY2025/26 with inflation remaining above 20%, and market disruptions have deepened food insecurity nationwide. The 2026 HNRP appeals for approximately \$890 million but reflects severely constrained funding and narrowed operational space, forcing agencies to prioritize only 2.6 million people with the most severe needs for life-saving support.

Key Insights

1. Myanmar's humanitarian crisis continues to deepen amid persistent conflict, economic collapse, and the lingering impacts of the March 2025 earthquake. An estimated 16 million people—nearly one in three citizens—are projected to require humanitarian assistance in 2026, as conflict-driven displacement, natural disasters, funding shortages, and humanitarian access constraints exacerbate vulnerabilities nationwide.

The 28 March 2025 earthquake remains a defining event in the humanitarian landscape. Official figures from SAC confirm 3,770 deaths and over 5,000 injuries, with the Early Recovery Cluster reporting over 24,200 houses damaged (24% completely destroyed), 4,857 apartments affected, and 132 health facilities damaged. An estimated 3.5 million tonnes of debris blocks roads and markets, with clearance impeded by shortages of heavy machinery (43%) and labour (40%). Nearly three-quarters of the most affected townships continue to face critical shelter and livelihood gaps—64% of communities reported job losses and 30–31% reported damage to crops and workplaces—while disrupted water systems (44% of groundwater sources contaminated) have fueled outbreaks of cholera and dengue.

2. Following the junta-run general elections held over December 2025 and January 2026, as well as ceasefires between the regime and some ethnic armed organizations in northern Shan, the number of armed conflict incidents has gradually decreased. Nonetheless, air and ground offensives continue in the Central Dry Zone, as well as Rakhine, Chin, Kayah, Kayin, and Kachin States. The new government's 100-day roadmap announced on 21 April 2026, which invites resistance groups to enter peace talks and to surrender, have not been widely accepted.

Between January and November 2025, the military conducted 2,165 airstrikes, up from 1,716 in all of 2024. On 10 December 2025, a military airstrike destroyed Mrauk-U General Hospital in Rakhine State, killing at least 33 people and injuring 76–80—prompting UN, WHO, and Amnesty International condemnation and calls for investigation as a possible war crime.

IDPs have reached approximately 3.7 million people as of March 2025, with projections indicating it could rise further through 2026. The AA now controls all but three of Rakhine's 17 townships, while resistance forces have captured extensive border areas in the southeast. Attacks on healthcare are widespread: Insecurity Insight recorded 1,815 attacks on health care between February 2021 and November 2025, with WHO verifying 67 attacks in 2025 alone. Access restrictions, including the junta's blockade of Rakhine since late 2023, continue to endanger civilians—particularly Rohingya communities facing persistent protection risks, restricted mobility, and limited access to aid.

3. Humanitarian access remains severely limited by insecurity and deliberate obstruction. The 2026 HNRP appeals for approximately \$890 million but reflects severely constrained funding and narrowed operational space, prioritizing only 2.6 million people with the most severe needs at a cost of \$521 million.

Partners reached some 6.3 million people at the end of 2025, but access constraints—including roadblocks, landmines, telecommunications blackouts, and the Rakhine blockade affecting 600,000 IDPs—force reliance on low-profile, community-led delivery modalities. WFP warns that hunger has reached "unacceptable" levels, with over 400,000 young children and mothers surviving on nutrient-deprived diets of plain rice or watery porridge.

4. The Food Security Cluster estimates about 12 million people face moderate-to-severe food insecurity in 2026, driven by conflict, displacement, rising food and fuel prices, reduced market access and supply-chain disruptions. September 2025 analyses reported "Serious" to "Critical" Global Acute Malnutrition in multiple conflict-affected townships, notably in northern Rakhine, Kachin, Sagaing, southern Shan, and Kayah—with nutrition and treatment services suspended or operating at minimal capacity due to access constraints.

Economic conditions continue to deteriorate: the World Bank projects real GDP will contract 2.0% in FY2025/26, with inflation remaining above 20%. Rising poverty and fear of forced conscription have driven male migration, leaving women, girls, and older people as household heads. More than four in ten households identify lack of income as their top challenge, and 83% of internally displaced families can meet half or less of their basic needs. The convergence of conflict, disaster, and economic decline continues to erode coping capacities, pushing Myanmar further toward a protracted humanitarian emergency with no signs of de-escalation.

5. International condemnation has intensified following the 10 December 2025 airstrike on Mrauk-U Hospital. The UN High Commissioner for Human Rights called for an investigation, warning the attack could amount to a war crime. WHO's Director-General condemned the strike as among dozens of verified attacks on healthcare in 2025. Amnesty International and the Burma Human Rights Network have urged the UN Security Council to refer Myanmar to the International Criminal Court, impose an arms embargo, and authorize emergency cross-border humanitarian aid.

Peace negotiations remain effectively stalled: resistance leaders and the National Unity Government publicly rejected Chinese-brokered terms for northern Shan State. The junta cancelled voting in over 67 townships where it cannot secure control, while nationwide "silent strikes" signalled popular rejection of the military's December 2025 elections. EU and Australia have declared the vote "not credible" amid junta repression. The junta denied responsibility for civilian casualties, accusing "terrorists" of being targeted, while humanitarian organizations stated that the pre-election environment has driven further escalation rather than de-escalation.

6. After taking office on 10 April 2026, president Min Aung Hlaing announced a 100-day program inviting resistance groups — both NCA signatories and non-signatories, including People's Defense Force (PDF) units — to enter peace talks by 31 July 2026. However, the Karen National Union (KNU), All Burma Student's Democratic Front (ABSDF), and the Chin National Front (CNF) quickly rejected the offer, with the KNU stating it has no intention of negotiating under frameworks linked to military rule, underscoring the deep erosion of trust between the junta and resistance forces.

Economy

Current Economic Situation

Myanmar's bilateral trade with India was reported at approximately US\$1.95–2.0 billion in 2025–2026, with both governments agreeing to accelerate this toward short-term targets of US\$3 billion and eventually US\$5 billion [26][39][28][6]. Myanmar's leadership has sought investment in agriculture, timber, minerals—including rare earths—rubber, information technology and digital infrastructure [6][26]. To support trade, India and Myanmar agreed to facilitate bilateral transactions through a Rupee–Kyat settlement mechanism, which has recorded steady growth since its operationalisation in May 2024 [26]. India signalled it will continue engagement with Myanmar despite Western sanctions imposed after the 2021 coup, with Indian officials arguing sustained dialogue is preferable to isolation [48][28].

Myanmar's extractive sector remains a major global supplier of rare earths, copper, tin and gemstones, with significant Chinese investment and largely unregulated operations in conflict-affected areas, exposing the industry to operational and safety risks [34][21]. The junta introduced measures to encourage electric-vehicle imports—including trade-in schemes—amid heavy energy import dependence (approximately 90% of supply), while EV exemptions from alternate-day driving rules have pushed up prices and created market uncertainty for dealers [20]. Reports used a reference exchange rate of \$1 = 2,100 MMK as of mid-March 2026 for local conversion estimates [2].

Conflict Economic Landscape

President Min Aung Hlaing's May 30–June 3, 2026 visit to India underscored trade and investment ambitions, though analysts warned the visit is unlikely to generate significant new investment given ongoing insecurity and the fact that large areas remain outside central military control, constraining investor access and project delivery [6][26]. Both governments agreed to deepen cooperation through the Rupee–Kyat settlement mechanism and collaboration on agro-processing, petroleum, energy, and mining, as well as connectivity projects including the Kaladan Multi-Modal Transit Transport project and the India–Myanmar–Thailand Trilateral Highway—both of which remain delayed by security challenges in Myanmar [26][6][28]. Central bank leadership was included in the Myanmar delegation, suggesting coordination on monetary and trade matters at senior levels [26].

Governance and safety risks in the extractive sector were highlighted by a major explosives blast at a mining site in Namhkam, Shan State on 2 June 2026, underscoring unregulated operations and links to Chinese investors and armed groups in resource areas [34]. Conflict-driven disruptions have cut trade routes and supply chains and undermined livelihoods across the country. In Arakan (Rakhine) State, a strict blockade and transport restrictions have displaced some 600,000 people since renewed fighting, driving food price inflation and severely limiting market access [15]. New displacements in Tanintharyi of over 5,000 since mid-May 2026 have further compounded these pressures [8]. Inle Lake communities in southern Shan State remain unable to repair earthquake damage a year on, amid sharp cost increases—carpenter wages up to 45,000 MMK/day, a basic stilt house costing approximately 5 million MMK, and rice reaching 160,000 MMK per bag—alongside a severely depressed tourism sector [5].

Health and basic-service recovery remains highly uneven: health partners reported operating in only 106 of 227 prioritized townships (47%) and covering just 35% of the 86 townships classified as severity level 4, with 78% of high-severity health townships located in areas not under military control [12]. Overall, insecurity, fragmented territorial control, weak governance in resource sectors and constrained humanitarian funding are limiting investment, infrastructure reconstruction, and the restoration of livelihoods across conflict-affected parts of Myanmar [6][12][15].

Stability of Currency and Bank System

India and Myanmar agreed on June 1, 2026 to facilitate and enhance bilateral trade through a Rupee–Kyat settlement mechanism, noting steady growth in transactions since its operationalisation in May 2024 [26]. Bilateral trade was reported at over US\$2 billion in 2025–2026, underscoring the commercial rationale for closer currency settlement arrangements [28]. Central bank leadership was included in the Myanmar delegation to India in late May/early June 2026, suggesting coordination on monetary and trade matters at senior levels [26].

The junta's commerce ministry announced an electric vehicle (EV) import-for-surrendered-vehicle scheme on May 28, 2026, stating that the central bank would supply required foreign currency for permits at an "appropriate" exchange rate, indicating direct central bank involvement in the allocation of foreign exchange for policy-led imports [20]. An alternate-day driving rule introduced in March 2026, from which EVs were exempted, combined with demand pressures, contributed to sharp EV price increases, with some models reportedly reaching "hundreds of millions of kyat," reflecting market volatility [20]. Myanmar's heavy dependence on imported energy—approximately 90% of supply—was cited by authorities as a key driver for prioritising EV imports to reduce fuel reliance [20]. Reports used a mid-market exchange rate of \$1 = 2,100 MMK as of 18 March 2026 for conversion estimates [2].

Employment

Conflict-driven displacement has severely constrained livelihoods and employment across Myanmar. More than 3,000 internally displaced persons in southern Rakhine (Taungup, Thandwe and Gwa townships) face acute food insecurity and a near-total absence of casual labour or income [15]. Intensified operations since mid-May 2026 displaced over 5,000 people in Tanintharyi, and around 12,000 people remained displaced in western Bago since April 2026, limiting access to employment and basic services [8].

Local labour market data indicate sharp variability in wages and conditions. In the Inle Lake area of southern Shan State, carpenter daily wages rose to 45,000 MMK, while typical daily earnings were reported at approximately 8,000 MMK for women versus up to 150,000 MMK for men; declines in tourism, including widespread hotel closures, have further reduced local jobs [5].

Extractive and mining activities in conflict-affected areas operate largely informally under armed groups, generating revenue and employment but posing severe safety risks, as illustrated by a gelignite storage explosion in Namhkam that killed dozens and injured scores in late May/early June 2026 [25][34].

Attacks on health care have significantly weakened health-sector staffing and service coverage. Insecurity Insight recorded 1,929 incidents of violence against health care across Myanmar between 1 February 2021 and 12 May 2026, with at least 173 health workers killed and 930 arrested; 73 attacks were recorded between 1 January and 31 May 2026, while WHO verified 38 attacks in the same period [11][12]. Health partners reported providing services in only 106 of 227 prioritized townships (47%) as of Q1 2026 [12].

Politics

Governance Structures and Rule of Law

Myanmar's ruling junta continues to seek international recognition while consolidating power. Junta leader Min Aung Hlaing visited India and met Prime Minister Narendra Modi on 3 June 2026 to discuss trade, security, border management, and connectivity projects [98][116][109][96]. His delegation included prominent business figures and representatives from the Union of Myanmar Federation of Chambers of Commerce and Industry, highlighting close ties between the military leadership and private-sector cronies that raise transparency and anti-corruption concerns [98]. Bilateral talks covered cooperation on cybercrime and human trafficking, and Indian officials noted joint efforts that rescued over 2,400 Indian nationals from scam centres in the past 18 months [109]. The April 2026 parliamentary election was widely denounced as a sham that consolidated military control, after which Min Aung Hlaing assumed a civilian presidential role [102]. The National Unity Government (NUG) formally urged India to weigh the broader implications of engaging the junta, warning that such engagement could normalise military rule [109].

Armed governance fragmentation is pronounced in Rakhine State, where the Arakan Army states it controls almost the entire territory and reports that junta bombardments and airstrikes on civilian towns have increased, reflecting contested authority and weakened law-enforcement integrity [114]. Civil society monitors report that human rights abuses have worsened under the junta-turned administration, with intensified air and drone strikes on villages, schools, and religious buildings, alongside widespread arbitrary arrests and repression [82]. Political prisoners have died in detention due to lack of adequate medical care, with two reported deaths on 22–23 May 2026 [82]. The junta has tightened information controls, revoking the publishing licences of three media outlets on 11 May 2026 and arresting more than 200 social media users during the election period, according to independent press monitors [82].

Political Freedoms and Recent Laws

Since the 2021 coup, civil society monitors report intensifying arbitrary arrests, airstrikes, and repression, with human rights groups saying conditions have worsened rather than improved [82]. The Assistance Association for Political Prisoners (AAPP) reports the regime currently holds more than 22,000 people on political charges, while nearly 9,000 have been released [106]. Local monitoring groups recorded two political prisoner deaths in custody on 22 and 23 May 2026, highlighting detention-related medical neglect and abuses [82].

A Myanmar Internet Project analysis released in late May documented 374 arrests for online expression — 114 of those arrested were women — and describes systematic digital repression through shutdowns, surveillance, and weaponized laws that drive self-censorship and increase physical risks for those seeking signals in remote areas [87]. Authorities reportedly tracked down and arrested more than 200 social media users during the election period, further signaling an expanding crackdown on independent media and online dissent [82].

Women's rights monitors documented a rise in gendered violence linked to the conflict, including 44 women killed in April amid air and ground attacks by junta forces [82]. Sexual violence is being used as a documented tactic of political repression, forming part of a patterned repertoire of abuses against civilians and detainees [97].

Min Aung Hlaing's state visit to India from 30 May–3 June 2026 and related bilateral agreements — including a Rupee-Kyat trade settlement announced on 1 June — prompted criticism from Myanmar opposition figures and regional rights parliamentarians who warned the engagement risks legitimising the junta [109][91][69]. Overall, authorities are applying digital restrictions, punitive arrests, and administrative measures selectively to shrink civic space and inhibit political participation and independent journalism [87][82].

Influence of Non-State Actors and Political Factions

Since the 2021 coup, Myanmar's conflict has been highly fragmented, with numerous ethnic armed organisations exercising local control and parallel administration across large swathes of the country [76][105][104]. Several groups, including the Karen National Union, have treated pre-coup ceasefires as void, expanded territorial governance, and continue to press for a federal system, deepening fragmentation and complicating centralised authority [105][104]. In Shan State, groups such as the TNLA control territory and economic assets that fund and reinforce their local authority [129].

Both non-state armed groups and the Tatmadaw have increased their use of air power and armed drones, producing cross-border incidents and civilian deaths — including drone explosions on the Thai border and deadly incidents on the Bangladesh border on 24 May and 2 June 2026 [119][76][82]. In Rakhine (Arakan) State, the Arakan Army claims control of almost the entire territory, while a junta-imposed blockade and active fighting have contributed to roughly 600,000 people displaced since November 2023 and severe restrictions on humanitarian access [114][95]. Conflict-related sexual violence by both state and non-state actors is documented as a systematic tactic of political repression and intimidation [97].

Regional engagement with the military government — including high-level visits and security cooperation by neighbours such as India — sustains informal external ties that affect the junta's legitimacy and governance calculus [96][112].

Public Trust in State Institutions

Multiple patterns of junta conduct have been documented by civil society and local monitoring groups, pointing to a marked erosion of legitimacy and public confidence in state institutions. Civil society monitors report intensified junta violence — including daily airstrikes, drone strikes, ground assaults, arbitrary arrests, and attacks on villages, schools, and religious buildings — alongside the revocation of publishing licences by the Ministry of Information on 11 May 2026, indicating sustained repression of dissent and information flows [82][87].

Local rights groups recorded extensive civilian property destruction and displacement in May: the Karenni Human Rights Group reported more than 120 houses, vehicles, and other properties damaged in Karenni State, including around 106 buildings destroyed by arson in Hpruso Township and roughly 140 households (approximately 7,000 people) forced to flee during operations that month [127]. A mortar explosion on 2 June killed a child near the Bangladesh–Myanmar border in Bandarban's Naikhongchhari area, highlighting continued spillover insecurity from Myanmar's conflict [76]. These patterns — attacks on civilians and health services, digital repression, media shutdowns, and mass property destruction — collectively underscore the deep erosion of institutional legitimacy under the junta [82][87][127][123].

Role of Marginalized Groups

Marginalized communities — including women, ethnic minorities, and political detainees — face acute and compounding vulnerabilities under the junta's ongoing military operations. The military conducts near-daily airstrikes and drone strikes targeting villages, schools, and religious buildings, severely constraining the civic space of affected communities, particularly ethnic populations in conflict-affected areas [82].

Violence against women and girls has risen sharply, with the Burmese Women's Union documenting 44 women killed in April alone amid military operations [82]. Digital repression disproportionately affects women and activists: of the 374 people arrested for online expression documented by the Myanmar Internet Project, 114 were women, while communications blackouts force people to travel long distances to access signals, increasing physical risks [87]. Ethnic armed organisations, including the Karen National Union and groups in Shan State, continue to exert territorial and political influence, with minority communities caught between competing armed actors and the junta's forces [105][104][129]. Media suppression — including the revocation of publishing licences for multiple outlets in May — has further shrunk avenues for marginalised groups and civil society to participate in public life [82].

Security And Stability

Security Overview

Military operations — including frequent airstrikes and drone strikes alongside large-scale ground offensives — have intensified across multiple regions, causing increased civilian harm and displacement [148][183]. Since mid-May, operations in Tanintharyi Region displaced more than 5,000 people sheltering in forested areas with severe shortages of food and basic supplies, and in western Bago around 12,000 people remained displaced since April amid continued airstrikes and constrained humanitarian access [169]. A junta offensive and accompanying aid blockade along the Chin–Magway border forced over 14,000 civilians to flee into mountainous areas, where camp managers reported acute shortages of shelter, medical supplies and food and documented executions and other killings during May operations [171]. Active frontlines, aerial bombardment and reported aid blockades continue to constrain humanitarian access and elevate protection risks for civilians and aid workers [171][148][169].

Civil society groups on 2 June urged Japanese companies KDDI and Sumitomo to end partnerships with junta-linked Myanma Posts and Telecommunications (MPT) over concerns that MPT's networks enable mass digital surveillance used by the junta to track and repress civilians [172]. The junta leader's state visit to India (30 May–3 June) drew condemnation from ASEAN parliamentarians and rights groups, who warned the visit risks legitimising the junta and underscored regional political tensions [173][162].

Security Forces and Law Enforcement

Intensified military operations, including airstrikes and drone attacks in Chin, Magway, Rakhine, Sagaing, Kachin, Karenni and northern Shan, have severely impeded access to health care and the transport of medical supplies [138]. Between 1 January and 31 May 2026, Insecurity Insight recorded 73 attacks on health care while WHO's Surveillance System for Attacks on Health Care verified 38 incidents, with heavy weapons accounting for the largest share of incidents (approximately 70% in 2025 and 74% in the first five months of 2026), indicating sustained use of heavy force against health infrastructure [145]. Since the 2021 coup, Insecurity Insight documented more than 1,929 incidents of violence against health care up to 12 May 2026, with 66% of attacks attributed to the Myanmar Armed Forces, at least 173 health workers killed, 930 arrested and health facilities damaged on at least 503 occasions [139]. Specific incidents recorded in May 2026 include a 19 May landmine/ambulance attack that damaged an LNGO ambulance and killed an LNGO worker, and artillery damage to two station hospital buildings in Mandalay on 24 May 2026 [134].

The Myanmar Health Cluster reports that health partners operated in 106 of 227 prioritized townships (47%) in Q1 2026, and that only 30 of 86 severity level 4 townships (35%) had partner interventions, with 78% of high-severity townships located in areas not under Myanmar military control [145]. Ongoing registration restrictions imposed by de facto authorities, together with intensifying conflict, are forcing some humanitarian partners to cease activities and are severely constraining service delivery and accessibility in rural and high-severity areas outside urban centres [145].

Myanmar's leadership engaged externally in late May–early June 2026, with President Min Aung Hlaing meeting Indian leaders to discuss border security and pledging that Myanmar territory would not be used against India's security interests, while India noted cooperation on repatriating over 2,400 nationals from cyber-scam centres in Myanmar [136][141]. The junta's digital repression and surveillance ecosystem has driven prosecutions and detentions for online activity, with documented arrests totalling 374 people, of whom 114 were women in the reported period, further undermining civil liberties and enabling targeted enforcement actions [144][172].

Monitoring mechanisms also report that sexual violence is being used as a tactic of political violence in Myanmar, reflecting grave protection concerns associated with security forces and other armed actors [166].

Demography

Demographic Overview

As of early June 2026, conflict-driven internal displacement is concentrated along the Chin–Magway border, where Mindat Township's displaced population rose to about 17,000 — including roughly 7,000 evacuated from Mindat town, 7,000 from neighbouring villages, and some 10,000 long-term IDPs — as of 2–3 June 2026 [190][189]. A regional snapshot for 26 May–1 June 2026 reported intensified operations in Tanintharyi displacing more than 5,000 people since mid-May, and about 12,000 people remaining displaced in western Bago since April, with humanitarian access constrained in those areas [191]. The humanitarian and health impacts of the conflict span diverse ethnic areas — including Chin, Kachin, Karen/Kayin, Karenni, Mon, Rakhine, Sagaing, Shan and Tanintharyi — reflecting multi-ethnic displacement and service gaps across both rural and border communities [184][193].

The Health Cluster reported that in Q1 2026, partners were providing health services in 106 of 227 townships prioritized in the 2026 Humanitarian Needs and Response Plan (47%). Of 86 townships categorized as 'extremely deprived' (severity level 4), partners reported interventions in only 30 (35%), noting that 78% of high-severity townships are located in areas not under Myanmar military control, which limits access [184].

UNHCR reported approximately 1.2 million Rohingya refugees living in Bangladesh, including some 150,000 arrivals since early 2024 and roughly 750,000 who fled during the August 2017 influx [212][201]. UNHCR and partners appealed for \$710.5 million for the Rohingya response, with the appeal around 60% funded at the time of reporting [201]. UN agencies noted that 2025 was the deadliest year on record for maritime departures by Rohingya, with nearly 900 people reported dead or missing in the Andaman Sea and Bay of Bengal, highlighting ongoing cross-border protection pressures [201][214]. UNHCR also reported in 2025 that more than 4.4 million people were stateless globally, with Myanmar among the countries hosting large stateless populations, reinforcing the regional dimensions of displacement and exclusion [211][212].

Socio-Cultural

Socio-Cultural Overview

Myanmar's social landscape is deeply fractured along ethnic, political, and geographic lines, with control contested between the military and multiple ethnic armed organisations and local People's Defence Forces. Mawdaung town had been under Karen National Union, Karen National Liberation Army, and People's Defence Force control for six months before being re-seized by the military on 19 May 2026 [258][232]. High-severity, health-deprived townships are disproportionately located in areas not under junta control—78% of severity level-4 health townships fall outside military control—limiting partner access and services in those communities [230]. Intensified offensives have generated large internal displacements along ethnically mixed frontlines: roughly 14,000 people fled the Chin–Magway border area in mid-May to early June 2026, including about 7,000 evacuees from Mindat town and 7,000 from neighbouring villages, with junta blockades and the monsoon aggravating acute shortages of shelter, food, and medicine, and widespread exposure to wet, moldy rations and inadequate roofing materials [226].

Rohingya camps and borderlands are reported to be embedded within wider conflict dynamics—armed recruitment, criminal networks, and spillover insecurity affect camp and host-community social cohesion and protection [266]. Civilians and civic institutions face targeted repression: junta operations include airstrikes and attacks on villages, schools, and religious buildings, and several independent media licences were revoked on 11 May 2026 [231]. Violence against health services is pervasive and erodes community resilience: Insecurity Insight recorded 1,929 incidents of violence against health care between 1 February 2021 and 12 May 2026, with 66% attributed to the Myanmar Armed Forces, at least 173 health workers killed, 930 arrested, and health facilities damaged on at least 503 occasions [232]. The growing use of armed drones has both targeted medical services and produced cross-border harm, including an errant drone blast that killed three Myanmar migrant workers in Thailand in early June 2026 [257]. Information and digital environments are highly restrictive and polarised, shaping public narratives about aid, displacement, and accountability and complicating humanitarian communication with affected communities [258]. Observers report that shared trauma from widespread atrocities is fostering socio-political alignment between the Bamar majority and ethnic minorities, strengthening a collective resistance identity that is reshaping communal relationships and claims to belonging [225].

Gender Dynamics

Civil-society monitoring indicates rising violence against women: the Burmese Women's Union documented 44 women killed in April 2026 from junta air and ground attacks, and reporting by Atrocity Alert and the UN notes that sexual violence in Myanmar is used as a tool of political violence and collective punishment, increasing stigma and barriers for survivors to access protection and survivor-centred services [231][261]. Between 1 January and 31 May 2026, Insecurity Insight recorded 73 attacks on health care and WHO's Surveillance System for Attacks on Health Care verified 38 attacks in the same period [230][235]. Insecurity Insight has also documented at least 53 maternal health services damaged, disrupted, or otherwise affected, reducing access to care for pregnant women, mothers, and newborns [232]. On 1 June 2026, UNFPA reported organising two Clinical Management of Rape (CMR) capacity-building sessions and, together with PATH, developing a 2026–2030 Family Planning Investment Case and conducting family-planning awareness activities to address increasing reproductive health needs [230]. The Myanmar Internet Project found 114 women among 374 people arrested for online expression in its dataset and warned that internet shutdowns force women to travel to remote locations to access signals, heightening risks of violence and gender-based discrimination [244].

Impact of Socio-Cultural Drivers on Aid

The junta's intensified digital repression—internet shutdowns, surveillance, and weaponised laws—has generated widespread anxiety, prompted self-censorship, and forced people in areas such as Thandwe and parts of Rakhine to travel long distances to reconnect, undermining safe and consistent communication channels for humanitarian engagement [244]. Insecurity Insight monitoring from 13–26 May 2026 shows contested social-media narratives and growing mistrust toward aid actors—with community expressions such as "We Haven't Received A Single Bottle Of Water" and "The UN Shouldn't Even Exist Anymore"—signalling damaged community trust and risks to the perceived neutrality and accountability of humanitarian messaging [258]. Sharp funding cuts and constrained access for Rohingya refugees and host communities are leaving vulnerable groups—including women and girls, persons with disabilities, and older people—highly dependent on aid and increasing tensions between host and displaced populations [260][264][267]. Cross-border incidents, including landmine explosions on 24 May and a deadly mortar on 2 June, reflect how borderlands have become contested spaces controlled by multiple armed actors, eroding state governance and complicating humanitarian access, local coordination, and acceptance of external responders [266]. Local socio-economic dynamics—such as gendered wage disparities and repeated natural hazards in areas like Inle Lake that have forced households to prioritise daily survival over rebuilding—alter informal support networks and influence how communities engage with and depend on humanitarian assistance [286].

Environment

Environmental Conditions

Myanmar's 2026 monsoon had a late onset, leaving acute watery diarrhoea (AWD) cases lower than the same period in 2025 [319]. Hydrometeorological forecasts in early June 2026 warned of heavy thunderstorms and flood risk, with GLOFAS and Myanmar's Department of Meteorology and Hydrology flagging Yangon and Bago states as particularly at risk, including scenarios up to 1-in-20-year flood events [320]. WMO seasonal outlooks indicate a pronounced shift toward El Niño for June–August 2026 (around 80% probability), with the Maritime Continent expected drier-than-normal — a regional climate driver likely to influence Myanmar's precipitation patterns and extreme-weather risk [320]. Parts of Myanmar experienced severe haze from land and forest fires in early 2026, peaking in March, contributing to transboundary air-quality challenges and increased respiratory risk for vulnerable populations in the Mekong sub-region [303]. In Inle Lake (Nyaungshwe Township), the lake level rose by about three inches with the onset of the rainy season and sediment accumulation at the lake bed is increasing local flood risk to plantations and settlements — compounded by unrepaired damage from the 28 March 2025 earthquake [333]. Overcrowding in temporary settlements during the rainy season was highlighted as increasing risk of skin and water-related diseases, reinforcing the need for WASH interventions [319].

Environmental Challenges

Localized AWD/cholera outbreaks were detected in Karen, Karenni, Mon, Sagaing and Southern Shan, driven by chronic poor hygiene and contaminated drinking water [319]. Malaria surges were also reported in Chin, Kachin and Tanintharyi, driven by poor WASH conditions, limited diagnostics and insufficient vector control [319]. Monsoon rains in May–June compounded humanitarian needs: a junta offensive combined with torrential rains displaced roughly 14,000 people around Mindat in early June 2026, leaving displaced families exposed and lacking shelter, food and medical supplies amid a junta blockade of supply routes since 14 May [289]. Intensified fighting, heavy bombardments and airstrikes have damaged towns, driven displacement and contributed to broader environmental degradation in conflict-affected areas, complicating access for environmental and WASH responses [338][341]. The Health Cluster warned that conflict and access constraints — with most high-severity health townships outside junta control — combined with transport bans (e.g., medicines blocked at Pakokku) and deliberate blockades are severely limiting preparedness, anticipatory action and response capacity [319][300][289].

Environmental Risks and Mitigation

GLOFAS and NOAA/FEWSNET forecasts in June 2026 indicated up to 300 mm of rainfall in a 10-day period and a risk of up to 1-in-20-year flooding for Yangon and Bago states, while the Department of Meteorology forecast thunderstorms during the first 10 days of June — raising the risk of flash floods and landslides flagged by regional risk monitors and the Start Fund [320][317]. Inle Lake communities face recurring seismic and hydrometeorological hazards, including repeated floods, droughts and windstorms, with earthquake damage from March 2025 remaining largely unrepaired [333]. In response to these compounding risks, Health and WASH partners have undertaken joint AWD action planning, developed a Township Prioritization Tool, and are expanding prepositioning of medical supplies for rapid deployment to hard-to-reach areas [319][290]. UNICEF organized an AWD preparedness workshop in Myitkyina on 4–5 June 2026, and updated AWD action plans have been developed for Northwest, Karenni, Shan, Rakhine and Southeast regions [319][290].

Conflict

Overview of the Ongoing Armed Conflict

Since mid-May through early June 2026, active hostilities have been reported across multiple areas of Myanmar — notably northern Shan (Namhkam), Rakhine (Kyaukphyu/Ramree), Chin (Mindat), Tanintharyi and western Bago — producing new displacement and civilian harm [390][379][370][388][378][367][359]. A massive detonation of stored mining explosives in Namhkam (northern Shan) around 31 May–1 June killed dozens (reports range from at least 39 up to 55), injured many and destroyed scores of homes, with casualty and damage figures varying between sources [390][379][370][388][380]. Junta air and naval strikes and ground fighting around Kyaukphyu intensified at end-May/early June, with airstrikes on 28 May and 1 June reported to have hit IDP shelter sites near Mount Peinhne, killing and injuring civilians and forcing thousands from over 30 villages to flee into forested areas [378].

Fierce fighting and junta counter-operations in Chin State have driven large new displacement into Mindat Township, where local actors estimated about 17,000 people displaced as of 2 June 2026 [367].

Conflict intensity is marked by frequent airstrikes and rising drone use, and attacks on health care are widespread. Insecurity Insight recorded 1,929 incidents of violence against health care from 1 February 2021 to 12 May 2026, attributing 66% to the Myanmar Armed Forces and documenting at least 173 health workers killed, 930 arrested, 503 health facilities damaged and more than 80 drone-related incidents affecting health services [352][346]. Specific May 2026 incidents include an LNGO ambulance damaged by a landmine on 19 May that killed an NGO worker and injured others, hospitals and clinics being shelled or militarised, and destruction of medical infrastructure undermining service delivery and disease surveillance [352].

Cross-border spillover risks have increased, including an errant combat drone that killed three Myanmar migrant workers in Thailand and recurring stray-ordnance incidents affecting Bangladeshi border communities [401][386]. Humanitarian access remains severely constrained by active hostilities, blockades and insecurity, and UNHCR and humanitarian partners have warned that funding cuts and access constraints are worsening conditions for Rohingya refugees and newly displaced populations in Rakhine and other areas [376][391][378][359].

Conflict Stakeholders

The Myanmar military (Tatmadaw/junta/State Security and Peace Commission) remains the principal state actor, conducting intensified ground offensives and coordinated aerial bombardments since leadership changes in March–April 2026, pursuing consolidation of territorial control and international re-engagement [409][405]. It increasingly relies on airpower, sieges, forced conscripts and large-scale offensives to hold towns and supply routes, while its overall combat capability is gradually degrading [369][352][409][356].

Ethnic armed organisations and resistance forces — notably the Ta'ang National Liberation Army (TNLA), Arakan Army (AA), Myanmar National Democratic Alliance Army (MNDAA), Kachin Independence Army (KIA), Karen National Union (KNU), and local People's Defence Forces (PDF) aligned with the National Unity Government (NUG) — control contested territories and conduct sustained operations against the junta using decentralized guerrilla tactics, drones and localized administration to erode junta supply lines [395][379][357][402][389][369]. The TNLA, a member of the Three Brotherhood Alliance, controls parts of northern Shan State including Namhkam and relies on mining revenues; an accidental detonation of stored gelignite in late May/early June 2026 highlighted the risks of rebel-controlled, largely unregulated extractive operations [379][370][395]. The Arakan Army exerts de facto control over most of Rakhine and has been generating diversified revenue for operations and services (e.g., a recently reported lottery) [379][399]. More broadly, non-state armed groups and local militias use resource extraction, artisanal mining and trade networks to finance operations, giving them significant economic leverage over territory and local populations [379][395]. Factional fragmentation, shifting local alliances, and the economic importance of resource-rich borderlands sustain a fragmented battlefield in which non-state actors wield substantial territorial governance and revenue-generation power [395][379][357].

China is an active external broker, having mediated ceasefires and exercising influence across parties, while India recently engaged the junta at the highest level — hosting junta leader Min Aung Hlaing on a state visit in late May/early June 2026 — to deepen trade, defence and border-security cooperation, reflecting competing regional external involvements [395][405][402]. The military's entanglement with illicit economies and the role of Border Guard Forces and allied militias in facilitating cyber-scams and organised criminal networks further amplify risks of transnational crime and prolonged instability [356].

Peace Negotiations

Mediation efforts remain fragmented and ad hoc, with no clear, sustained or inclusive negotiation process yielding a durable settlement [347]. ASEAN's Five-Point Consensus remains the official regional framework but has delivered little progress since the 2021 coup, with limited implementation of its provisions and slow movement on the special-envoy process [347]. The Philippines' ASEAN special envoy, Theresa Lazaro, visited Naypyitaw in early January, and Manila formally urged Myanmar on 6 May to allow the envoy to meet detained leader Aung San Suu Kyi as part of broader engagement efforts [347]. India hosted junta leader Min Aung Hlaing and reported that Suu Kyi's detention was raised "largely in the context of the peace process," with Indian officials stressing inclusion of all stakeholders for an enduring peace [364]. Separately, Timor-Leste President José Ramos-Horta proposed convening the Tatmadaw, ethnic armed organisations and the democratic opposition "under a tent," guided by a retired Southeast Asian four-star general, to seek a humanitarian solution and allow unrestricted humanitarian access [347].

Conflict Outlook

A sustained negotiated settlement appears unlikely in the near term, with a protracted war of attrition more probable than rapid de-escalation [356]. While the junta controls some major towns in Chin and Sagaing, its combat capability is degrading as it relies increasingly on indiscriminate airstrikes and expendable infantry, while resistance forces continually sever military supply and support lines [356]. The conflict has become a testing ground for drones, electronic warfare and low-cost decentralized combat tactics, expanding the reach of hostilities and raising risks to civilians and humanitarian actors [350][351].

Cross-border spillovers are intensifying regional instability: violence, deaths and abductions along the Bangladesh–Myanmar frontier, collapse of effective governance on the Myanmar side, and disrupted trade routes are straining bilateral relations and exposing border communities to greater harm [386]. Humanitarian access is increasingly constrained by insecurity and new warfare tactics, driving rising internal displacement, complicating Rohingya repatriation prospects, and leaving large numbers of border and internally displaced populations exposed to severe protection and assistance gaps [386][354].

Displacement

Displacement Overview

Recent reports document multiple new and ongoing displacement events across Myanmar in late May–early June 2026, driven by intensified military operations, airstrikes, and blockade tactics [447][440]. A major junta offensive launched in mid-May displaced roughly 14,000 people who fled into forests and surrounding areas around Mindat town, with 7,000 evacuated from Mindat town and around 7,000 from 13 villages in Saw Township joining approximately 10,000 long-term IDPs, bringing the township's total displaced population to about 17,000 [448][438]. Clashes were reported to be ongoing into early June, with displaced families facing severe monsoon exposure and acute shortages due to a junta-imposed blockade in place since 14 May [448][438]. Intensified operations since mid-May also reportedly displaced more than 5,000 people in Tanintharyi Region, while western Bago Region has around 12,000 people displaced since April amid continued airstrikes and access constraints [453]. In southern Rakhine State, more than 3,000 IDPs across Taungup, Thandwe, and Gwa townships face acute food insecurity and minimal humanitarian assistance [445]. Military operations in Hpruso Township between March and May involved the burning of over 80 homes and forced villagers to flee into forests, creating urgent shelter and food needs for newly displaced civilians [456].

Attacks on health care and the recurrent use of drones and heavy weapons have degraded health service access and are compounding displacement risks and protection concerns across multiple states [440][447]. Health Cluster reporting highlights overcrowding in temporary settlements, elevated risks of AWD/cholera and malaria during the monsoon, and ongoing AWD preparedness and prepositioning actions in early June [441][447].

Living Conditions of the Displaced Population

Displaced families in Mindat Township are facing acute shortages of shelter, food, and medicine, exposed to torrential monsoon rains, with reports of moldy rations, lack of tarpaulins, and a junta-imposed blockade disrupting transport of essential goods since mid-May [448]. IDP camps in Karenni State have reported surges in malaria, dengue, respiratory illnesses, and skin diseases since the onset of the rainy season, with the Interim Executive Council reporting 92,577 clinic visits and more than 10,000 malaria cases across four camps; clinics face medicine shortages and some health teams visit camps only quarterly [421]. In southern Rakhine State, more than 3,000 IDPs sheltering in forested or mountainous areas and makeshift camps across Taungup, Thandwe, and Gwa townships face acute food insecurity, rising food prices, increasing child begging, and a near-absence of humanitarian assistance ahead of the monsoon [445]. Overcrowding and WASH gaps in temporary settlements are elevating disease risks, and Health and WASH clusters have been updating AWD and measles preparedness plans while UNFPA prepositioned reproductive health kits to support service continuity in late May–early June 2026 [441].

Violence against health care has been extensive and persistent: between 1 February 2021 and 12 May 2026 there were 1,929 recorded attacks on health care (66% attributed to the Myanmar Armed Forces), with at least 173 health workers killed, 930 arrested, and health facilities damaged on at least 503 occasions, further undermining displaced populations' access to care [440]. As of May 2026, health services reached only 106 of 227 prioritized townships (47%) and 30 of 86 townships classified as extremely deprived of services (35%), with intensified conflict, restricted NGO registration, and access constraints limiting service delivery in high-severity areas [441].

Drivers of Displacement

Conflict and political repression by the Myanmar military — including large-scale offensives, airstrikes, the "Four Cuts" strategy, and deliberate blockades — are the primary drivers of displacement and severe access constraints for civilians and humanitarian actors [438][448][429][441]. Junta-imposed blockades have cut transport of food, medicine, and essential supplies from Pakokku toward western Yaw region townships since 14 May, aggravating shortages and humanitarian needs for displaced populations [448]. Monsoon rains and recurring natural hazards are compounding displacement impacts, with torrential rains worsening shelter, food, and health conditions for those hiding in forests or informal sites [448][453]. Conflict-related sexual violence has also been documented as part of the repertoire of political violence used in Myanmar, increasing protection risks for civilians [454].

Health service delivery is highly constrained by intensified clashes: partners reported providing services in 106 of 227 prioritized townships (47%), with interventions submitted from only 30 of 86 townships categorized as severity level 4 (35%), and 78% of high-severity health townships lying in areas not under military control [441].

Separately, targeted violence, persecution, and statelessness continue to drive Rohingya displacement. Roughly 150,000 new arrivals fled renewed violence in Rakhine since early 2024, 2025 saw nearly 900 Rohingya reported missing or dead at sea, and prospects for safe, voluntary return have faded [426][428][433]. These combined drivers — armed offensives, targeting and persecution, access-denying blockades, seasonal hazards, and chronic displacement dynamics — are producing repeated and secondary displacement, severely constraining humanitarian access and eroding prospects for safe return [448][441][426].

Integration

National and host-country policy constraints remain major institutional barriers to durable local integration for displaced populations. Bangladesh maintains political limits on formal integration, and Malaysia is not a party to the 1951 Refugee Convention, while junta restrictions and blockades inside Myanmar further foreclose options for safe return [418][450][472].

The wider Rakhine displacement since November 2023 totals some 600,000 people, and in southern Rakhine State more than 3,000 IDPs across Taungup, Thandwe, and Gwa townships face virtually no employment opportunities and inadequate shelter [445].

Inside Myanmar, intensified fighting around Mindat in early June 2026 has driven roughly 17,000 people into displacement, with large-scale evacuations and acute shortages of shelter, food, and medicines compounded by a junta-imposed blockade and ongoing monsoon conditions severely limiting any prospect of local stabilization or return [438][448].

Humanitarian Access

Humanitarian Access Overview

Health partners reported activity in 106 of 227 prioritized townships (47%) and interventions in only 30 of 86 'severity level 4' townships (35%), with 78% of high-severity townships located outside Myanmar military control, which fundamentally limits access [485]. Intensified armed clashes and NGO registration difficulties imposed by de facto authorities are significantly reducing partners' access, reporting capacity, and ability to operate in high-severity areas, with some partners forced to cease activities entirely due to registration barriers, while persistent internet connectivity and data-management constraints further limit reporting [485]. Violence against health care is widespread: Insecurity Insight recorded more than 1,929 incidents between 1 February 2021 and 12 May 2026 — including at least 173 health workers killed, 930 arrested, 503 health facilities damaged, at least 53 maternal health services damaged or disrupted, and over 80 incidents involving drones — with 66% of attacks attributed to the Myanmar Armed Forces [497][498]. Recent operational risks include a 19 May 2026 landmine-damaged LNGO ambulance on the Leiktho–Toungoo highway that killed one LNGO worker and injured others, and artillery damage to station hospital buildings in Natogyi township on 24 May 2026 [498]. Blockades and supply-chain disruptions in Rakhine/Arakan have left many IDPs with minimal or no assistance and acute food insecurity in southern Arakan townships where only a few aid groups operate [487][498]. Displacement and frontline advances continue to constrain access, with Mindat Township displacement rising to around 17,000 as of 2 June 2026 amid junta counter-offensives, increasing urgent needs and complicating humanitarian delivery [510]. In response, the Health Cluster is expanding prepositioning of medical supplies for rapid deployment to hard-to-reach areas, and the WASH Cluster developed a Township Prioritization Tool to be tested in Kachin during an AWD preparedness workshop on 4–5 June 2026, while AWD action plans are being updated for Northwest, Karenni and Shan, Rakhine, and Southeast regions [485]. Sexual and reproductive health services are being delivered mainly through mobile and community-based approaches, and UNFPA released 29 Inter-Agency Reproductive Health kits to partners to support CEmONC and referral services [485].

Barriers to Humanitarian Access

Access constraints vary significantly by region. In southern Rakhine (Taungup, Thandwe, Gwa), severe shortages of assistance and acute food insecurity affect over 3,000 IDPs, with junta-imposed blockades and a strict blockade in Arakan having severely disrupted transportation and supplies since renewed fighting [487][479][498]. More than 14,000 civilians fled amid a junta offensive and aid blockade along the Chin–Magway border, and multiple IDP camps in Karenni face clinic shortages and rising malaria and dengue risk due to infrequent health visits and limited medical supplies [479][480]. Reported bans at military checkpoints on transporting medicines and food from Pakokku further constrain humanitarian deliveries across affected areas [498].

Humanitarian Access Gaps

The most critical access gaps are concentrated among populations in high-severity townships, displacement sites, and refugee communities where the combination of active hostilities, targeted attacks on health infrastructure, blocked routes, and funding shortfalls prevents timely delivery of food, water, shelter, and health services [497][498][482][489]. Inside Myanmar, populations in southern Rakhine townships, along the Chin–Magway border, and in Karenni IDP camps face the greatest difficulty accessing humanitarian assistance due to the overlapping effects of insecurity, blockades, infrastructure damage, and insufficient partner presence in areas outside military control [487][479][480][485]. Drone threats against health facilities represent an emerging and growing operational gap, with over 80 such incidents already recorded, further restricting the ability of mobile and community-based teams to safely reach underserved populations [497][498].

Information, Communication and AAP

Information Landscape

Information about Myanmar in early June 2026 is drawn from UN agencies, local monitoring groups, and open-source incident trackers, producing partial overlaps and differing counts that require cross-validation for operational decision-making and response prioritization [544][578][576]. Between 1 January and 31 May 2026, Insecurity Insight recorded 73 attacks on health care in Myanmar, while WHO's Surveillance System for Attacks on Health Care (SSA) verified 38 attacks in the same period [544]. Analysis of incident types shows heavy weapons caused 70% of verified attacks in 2025 and 74% during January–May 2026, indicating persistence of high-intensity threats to health services [544]. Since the 2021 coup, Insecurity Insight has recorded 1,929 incidents targeting health care, including at least 173 health workers killed, 930 arrested, and more than 503 health facilities damaged [535][534].

Community-based monitoring by KHRG, using trained local researchers reporting in March–April 2026, provides granular, redacted incident narratives for human rights documentation and operational awareness, but censors names and precise locations for security reasons [578][575][571]. Forecasting and anticipatory systems flagged heightened flood risk for Myanmar in early–mid June 2026, with Start Network/Start Fund citing FEWSNET, NOAA, GLOFAS, and Myanmar's Department of Meteorology and Hydrology to encourage anticipatory alerts and actions [551][562].

Communication Channels

Humanitarian actors and affected populations rely primarily on mobile/internet-based platforms, social media and other digital platforms, and mobile- and community-based face-to-face approaches for information exchange [529][576][544]. Mobile networks are highly compromised: civil society accuses the state-linked operator Myanma Posts and Telecommunications (MPT) of enabling mass surveillance, censorship, and hosting junta propaganda sites, and companies linked to MPT were urged to divest in early June 2026 [529][526]. Internet shutdowns, pervasive surveillance, and frequent connectivity outages force people to travel long distances — over 10 miles and multi-hour hikes in some areas — to find a signal, impeding remote reporting by partners and undermining accessibility and reliability [526][544].

Digital platforms and social media are widely used for narratives around aid and displacement and are actively monitored by organisations, but online expression is dangerous: large-scale arrests for online activity have been documented, and propaganda and misinformation risks have been noted in monitoring briefs [576][526][529]. Face-to-face and community channels remain important – most sexual and reproductive health (SRH) services are delivered via mobile and community-based approaches – but intensified conflict and access constraints limit partner presence [544]. Health partners reported service delivery in only 106 of 227 prioritized townships (47%), and just 30 of 86 'severity level 4' townships had partner intervention data in Q1 2026 [544]. Timeliness and accuracy of information are further weakened by verification delays and restrictive information controls; the Health Cluster has highlighted ongoing reporting challenges while planning SSA awareness sessions to increase direct reporting of attacks on health care [576][535][544][534].

Barriers to Information

Access to information in Myanmar faces overlapping structural, security, and logistical barriers. The Myanmar Health Cluster reported persistent internet connectivity problems and limited data-management capacity among partners in May–June 2026, undermining timely reporting and information-sharing [544][534]. Ongoing issues with NGO registration by de facto authorities have forced some partners to cease activities entirely, further restricting formal channels for public information and aid communication [544]. Additionally, bans on transporting medicines at military checkpoints compound access constraints alongside intensified armed clashes [544][535][534].

Sustained violence against health services has created a climate of fear that hinders movement, reporting, and access to service information. A 19 May 2026 incident on the Leiktho–Toungoo highway damaged an LNGO ambulance and killed a staff member, illustrating how security incidents directly disrupt information flows and service delivery [576]. More than 80 drone-related incidents have been recorded, reflecting an intensification of aerial threats [535]. Forecast heavy rains and flood risk in early June 2026 for Yangon and Bago states are likely to create additional physical barriers to information access and aid delivery during the monsoon period [551][562].

The Health Cluster is planning SSA awareness sessions and other measures to encourage reporting, but access constraints and security risks continue to limit effective information dissemination [544][534].

Misinformation and Rumors

Within Myanmar-related humanitarian reporting, differences between open-source and verified data create additional verification challenges: the Health Cluster noted 73 attacks on health care reported by Insecurity Insight between 1 January and 31 May 2026 versus 38 verified by WHO's SSA system, underscoring the risks of unverified reports shaping perceptions of health access and response capacity [534][544]. To strengthen verification and trusted reporting, the Health Cluster planned SSA awareness sessions starting 1 June 2026 to train and incentivize partners to report attacks on health care directly to the WHO SSA system [544].

Accountability Needs and Transparency

Accountability for the humanitarian response in Myanmar is constrained by significant coverage gaps and operational barriers. As of Q1 2026, health partners reported providing services in 106 of 227 townships (47%) prioritized in the 2026 Humanitarian Needs and Response Plan, and in only 30 of 86 townships categorized as severity level 4 (35%), reflecting major gaps in the highest-need areas [544]. Constraints include intensified armed clashes, bans on transporting medicines at military checkpoints, NGO registration requirements by de facto authorities that have forced some organizations to cease activities, and persistent internet and data-management problems that limit partner reporting and community feedback [544][535][534].

The Health Cluster is taking steps to strengthen reporting and preparedness: conducting SSA awareness sessions to encourage reporting of attacks, developing a Township Prioritization Tool, updating AWD Readiness Action Plans, expanding prepositioning of medical supplies, and planning localization and risk-sharing workshops in mid-June to support partner reporting and risk mitigation [544].

Legal and Policy

Legal and Policy Framework

The legal and policy environment governing the Myanmar crisis is shaped by both domestic repression and contested international engagement. The junta's Ministry of Information revoked publishing licences for three media outlets on 11 May 2026, and restrictions on digital freedoms continue to tighten under cybersecurity and election-related laws that civil society says have led to prosecutions of hundreds of people [606][632]. Civil society organisations also publicly urged companies on 2 June 2026 to end partnerships with state-linked telecom operator MPT over its role in enabling mass surveillance and censorship, framing this as a legal and human-rights risk linked to corporate complicity in repression [632]. The junta-imposed blockade on Arakan (Rakhine) State since November 2023 has severely disrupted transportation and supply chains, undermining livelihoods and sharply limiting humanitarian access to displaced populations [612].

Key documented patterns of non-compliance with protections for health care include the following figures:

- 1,929 recorded incidents of violence against health care in Myanmar between 1 February 2021 and 12 May 2026, with 66% attributed to the Myanmar Armed Forces [600][603][605].
- At least 173 health workers killed and 930 arrested [600][603][605].
- Health facilities damaged on at least 503 occasions and 80+ recorded drone incidents affecting health services [600][603][605].

UNHCR data also place Myanmar among countries with large stateless populations in the Asia-Pacific, reinforcing chronic barriers to legal identity, documentation and access to services for affected groups [611].

Regionally, Timor-Leste's president and others have urged renewed mediation under ASEAN mechanisms, noting the Five-Point Consensus and debates over appointing an ASEAN-led mediator or special envoy to facilitate humanitarian access and dialogue [618]. India hosted Myanmar junta leader Min Aung Hlaing on an official visit from 30 May–3 June 2026, a move condemned by ASEAN Parliamentarians for Human Rights as legitimising the junta and flagged in a 28 May 2026 letter from the National Unity Government to India's foreign minister [609][626][637].

Humanitarian Impact of Legislation

Legislation and administrative policies imposed by Myanmar's de facto authorities have created severe operational constraints for humanitarian actors. NGO registration requirements and opaque accreditation processes have forced some organisations to cease activities entirely [604]. As of May 2026, health partners were providing services in 106 of 227 prioritised townships (47%) and in only 30 of 86 townships rated severity level 4 (35%), a gap explained in part by the fact that 78% of the highest-severity health townships are located in areas not under Myanmar military control, which reduces partner access and reporting capacity [604].

Direct obstructions to medical movements compound these barriers: junta checkpoints banned the transport of medicines from Pakokku on 13 May 2026, and an LNGO ambulance hit a landmine on 19 May 2026 near Chauk Maing village (Leiktho township) — despite having obtained junta clearance — killing one LNGO worker and injuring others, indicating ineffective or unreliable safe-passage mechanisms [603][600]. Artillery shelling also damaged functioning hospital buildings on 24 May 2026 in Natogyi township, further illustrating direct threats to humanitarian personnel, assets and facilities [600][603]. Between 1 January and 31 May 2026, Insecurity Insight recorded 73 attacks on health care while WHO's Surveillance System for Attacks on Health Care verified 38 attacks, signalling widespread violence undermining delivery of medical services across multiple states and regions [604][605][603].

Media and digital controls imposed by the de facto authorities — including revoking publishing licences in May 2026 and enforcing cybersecurity and election-related laws — have driven arrests, internet shutdowns and pervasive surveillance, measures that impede communications, reporting and protection for civilians and aid workers [606][608]. More broadly, sanctions, counterterrorism measures and bureaucratic impediments are identified as drivers that further constrain humanitarian access, increase legal and operational risks for aid actors, and hinder principled delivery of assistance [638].

International Agreements Impacting the Crisis

The international response to the Myanmar crisis is shaped by a combination of bilateral engagements, multilateral mechanisms and targeted measures that have produced mixed results. ASEAN's Five-Point Consensus remains largely unimplemented since the 2021 coup, and regional mechanisms have struggled to progress meaningful mediation or ceasefire enforcement [618]. China has acted as a regional broker, mediating a ceasefire with the Three Brotherhood Alliance in early 2024, underscoring Beijing's ongoing role in conflict mediation dynamics [624].

India hosted Myanmar junta leader Min Aung Hlaing from 30 May–3 June 2026; Prime Minister Narendra Modi met him on 1 June and both sides issued a joint statement to deepen cooperation, including facilitation of trade via a Rupee-Kyat settlement mechanism, advancing connectivity projects, and increasing Mekong Ganga ICCR scholarships for Myanmar from 36 to 100 from 2026 onward [627][626][633]. Indian officials stated that continued engagement aimed to support stability and was "not intended to be a commentary on the internal political arrangements," arguing sustained dialogue is preferable to isolation [631][626][630]. The visit drew sharp condemnation from ASEAN Parliamentarians for Human Rights and multiple civil-society groups, who warned it risked legitimising the junta [609][630][626][637].

Western targeted measures against Myanmar's senior leadership predate 2026: the United States first sanctioned Min Aung Hlaing in 2019, the UK imposed measures in July 2020, and the EU has maintained and extended sanctions. The Gambia's ICJ genocide case and an Argentinian arrest warrant against Min Aung Hlaing were also reported in 2025 [639].

Global humanitarian funding shortfalls and donor cuts — including a reported 25% decline in total humanitarian funding in 2025 cited by OCHA — are constraining international response capacity to the Myanmar-related displacement and refugee situations [645].

Limitations of Current Policies

Existing legal and policy frameworks exhibit significant gaps that undermine humanitarian response, human rights protection and crisis resolution in Myanmar. Intensified conflict, daily airstrikes and growing drone use are severely impeding humanitarian access and health service delivery, with health partners covering only 106 of 227 priority townships (47%) and just 30 of 86 extremely deprived (severity level 4) townships (35%) as of early 2026 [604][605]. Administrative barriers imposed by de facto authorities — including opaque accreditation requirements — have forced some partners to cease activities, creating bureaucratic impediments to programme continuity and scale-up [604]. Safe-passage mechanisms have proven ineffective or unreliable, as demonstrated by the medicine transport ban at Pakokku checkpoints on 13 May 2026 and the landmine incident on 19 May 2026 that killed an LNGO worker despite prior junta clearance [603][600].

Information repression — including the revocation of publishing licences for three media outlets on 11 May 2026 and ongoing internet restrictions — undermines transparency, monitoring and accountability for abuses and humanitarian needs [606][608]. The absence of effective accountability mechanisms is further compounded by contradictory diplomatic moves: ASEAN's Five-Point Consensus remains unimplemented, while India's hosting of coup leader Min Aung Hlaing during 30 May–3 June 2026 was condemned by ASEAN Parliamentarians for Human Rights as legitimising the junta and inconsistent with democratic obligations [609][618].

Donor funding shortfalls are reducing operational scale and refugee support. A reported 25% decline in total humanitarian funding in 2025 cited by OCHA further narrows the space for principled, adequately resourced humanitarian action [645]. Collectively, these gaps — spanning access constraints, impunity, information repression and financing shortfalls — point to the need for coordinated policy reforms that strengthen humanitarian access guarantees, enforce existing international legal obligations, and sustain predictable funding for crisis-affected populations.

Food Security and Livelihoods

Access to Food and Market Functionality

Since mid-May, junta-imposed blockades and checkpoint restrictions have disrupted key supply routes and market access in western and central Myanmar, including a ban on transporting medicines and food from Pakokku reported on 13 May and a broader cutoff of transport toward western Yaw townships since 14 May [692][690][681]. Intensified fighting around Mindat and neighbouring areas triggered large-scale displacement in early June — approximately 7,000 people evacuated Mindat town and a further 7,000 from surrounding villages by 3 June, joining long-term IDPs and worsening local shortages as displaced families report moldy rations and acute lack of essentials [681]. In southern Rakhine (Arakan), more than 3,000 IDPs face acute food insecurity driven by severe shortages of humanitarian assistance, lack of income or casual labour, and sharply rising prices following a strict junta blockade that has severely disrupted transportation and supply chains [677]. Local aid actors report extremely limited operational presence in parts of southern Arakan — around three aid groups operating in some areas — and describe relief support as minimal to almost nonexistent when it reaches displaced communities [677]. Since the renewed outbreak of conflict in Arakan (Rakhine) in November 2023, roughly 600,000 people have been displaced and the junta-imposed blockade has severely disrupted transportation and supply chains, undermining local livelihoods and market access [677]. Commodity and basic food prices have climbed in conflict-affected areas, eroding affordability for displaced and host households and prompting negative coping, while armed groups' economic measures and blockades have been cited as drivers of higher living costs [677][700]. Intensified airstrikes, drone attacks and other violence across Chin, Magway, Rakhine, Sagaing and other states are impeding movement of goods and humanitarian supplies, constraining market functionality and limiting the ability of partners to replenish stocks or scale up distributions in high-severity areas [675][674]. Start Network and seasonal outlooks flagged flood risk in Myanmar for June 2026, which may further disrupt markets, transport routes and local food availability during the monsoon period [671].

Coping Mechanisms

Displaced households in southern Rakhine (Arakan) State face acute food shortages and minimal humanitarian support, with begging increasingly used as a survival strategy and children reported among those begging in local markets [677]. Displaced families hiding in the mountains along the Chin–Magway front have been reported eating moldy food rations and lack basic shelter materials such as tarpaulins, heightening exposure and reducing food and non-food coping options [681]. IDP camps in Karenni State are dependent on donor support for healthcare and nutrition, with medicine shortages, inadequate nutrition, and rising malaria and other disease burdens undermining household resilience and coping capacity [691]. Overcrowding in temporary settlements and limited WASH and medical supplies increase the risk of disease outbreaks, and AWD/cholera preparedness and WASH–health coordination activities — including an AWD preparedness workshop planned for Kachin on 4–5 June 2026 — are underway to mitigate these risks [674]. Rohingya refugees from Myanmar remain heavily reliant on humanitarian assistance in Bangladesh, with funding cuts and food-ration reductions deepening hardship and pushing some refugees to risky sea crossings; nearly 900 Rohingya were reported missing or killed at sea in 2025, reflecting extreme coping-driven mobility and protection risks [665][676].

Livelihood Disruption

IDPs in southern Arakan State face near-total loss of income and virtually no casual day labour, with many resorting to begging and lacking funds to buy shelter materials ahead of the monsoon [677]. In Karenni State, overcrowded IDP camps report rising infectious disease caseloads and limited income, with clinics overstretched — 92,577 patients reported by the Interim Executive Council — and families unable to afford transport or medicines [691]. Attacks on health care, including ambulance damage from a landmine and bans on transporting medicines and food in areas such as Pakokku, have further disrupted service delivery and access to livelihoods and essentials [690]. The Health Cluster reported ongoing NGO registration challenges and intensifying conflict that are impeding health services, supplies and partner operations as of 1 June 2026 [674]. Agricultural livelihoods are being hit by deliberate military actions, with reports of rice fields and other farming assets targeted, reducing farming income and food production capacity [701]. In tourist-dependent Inle Lake (Nyaungshwe, southern Shan State), earthquake damage from 28 March 2025, high repair costs and closed hotels have reduced tourism income while local costs and wages have shifted — carpenter wages up to 45,000 MMK/day, women's casual wages around 8,000 MMK/day, and a bag of rice up to 160,000 MMK — eroding household purchasing power [666]. UNHCR and UN partners warned that Rohingya refugees remain largely dependent on aid with limited opportunities to earn, and that funding shortfalls and restricted access are worsening livelihoods for refugees and host communities [665][676].

Impact on Agricultural Activities and Production

Military operations, airstrikes and ground fighting have driven displacement and acute shortages, disrupting rural livelihoods across multiple regions of Myanmar [686]. Key displacement figures reflecting the scale of agricultural disruption include more than 5,000 people displaced from several villages in Tanintharyi Region since mid-May 2026, many sheltering in forested areas and facing severe shortages of food and basic supplies, and around 12,000 people remaining displaced in western Bago Region since April 2026 amid continued airstrikes and constrained humanitarian access [686]. Blockades, transport disruptions and constrained humanitarian access have limited farmers' and traders' ability to reach fields and markets, reduced incomes from casual agricultural labour, and contributed to soaring food prices and shortages of basic agricultural goods in affected areas [677]. Regionally, rising fuel costs and global fertiliser shocks are increasing production and transport costs; World Vision notes El Niño and energy/fertiliser pressures across mainland Southeast Asia, including Myanmar, are reducing input use and weakening fish and agricultural livelihoods [663]. Petrol price spikes reported in regional coverage have exceeded 90% in the early months of the 2026 energy shock, further raising costs for farm mechanisation, irrigation and transport and likely constraining input access [680][663].

Severe haze and fires that peaked in March 2026 across parts of the Mekong sub-region — including areas of Myanmar — have also damaged agricultural areas and disrupted outdoor farming and fishing activities, compounding risks to upcoming planting and

harvest cycles [694].

Food and Livelihood Needs and Current Response

WFP restructured the Rohingya food response on 1 April 2026 by introducing a food insecurity-based Targeting and Prioritization Exercise (TPE) that adjusted monthly entitlements while maintaining assistance coverage for all households, supporting 1.13 million refugees in Cox's Bazar and 28,000 in Bhasan Char in April 2026 through general food assistance [660]. At TPE rollout WFP faced initial beneficiary dissatisfaction and a temporary drop in collection rates, but full coverage was restored by mid-April through coordination with UNHCR, authorities and community leaders [660]. In April 2026, WFP delivered nutrition services reaching 95,500 children (6–59 months) and 43,700 pregnant and breastfeeding women, and enrolled 4,710 children with moderate acute malnutrition into treatment programmes [660]. The UN and the Government of Bangladesh launched a \$710.5 million appeal for Rohingya refugees and host communities for 2026 — 26% lower than the previous year and currently around 60% funded — raising concerns that funding shortfalls will force hyper-prioritisation and reduce essential services [665][669]. Inside Myanmar, access constraints and limited relief have left many displaced populations facing acute food insecurity, with reports citing over 14,000 civilians displaced toward the Chin–Magway border with accounts of moldy food rations, more than 5,000 displaced in Tanintharyi since mid-May, and around 12,000 remaining displaced in western Bago since April, all reporting severe shortages of food and basic supplies [677][681][686]. Blockades, disrupted supply chains and frequent attacks on health services are undermining delivery of food, nutrition and livelihood assistance across conflict-affected areas, further restricting humanitarian access and preparedness [677][690]. Health and WASH clusters are scaling preparedness for monsoon risks by developing a Township Prioritization Tool, updating AWD/cholera action plans, expanding prepositioning of medical supplies, and UNFPA released 29 Inter-Agency Reproductive Health kits while SRH services continue via mobile and community-based approaches to reach underserved populations [674]. Coverage gaps remain significant: partners reported health services in only 106 of 227 prioritized townships (47%), and only 30 of 86 townships rated as 'extremely deprived' submitted intervention data, indicating major geographic and service shortfalls for life-saving health, nutrition and WASH interventions that underpin food security and livelihoods [674]. Persistent funding shortfalls and access constraints continue to limit the scale and effectiveness of food and livelihood assistance despite coordination efforts among WFP, UNHCR, local authorities and cluster partners [660][665][674].

Education

Learning Environment and Facility Conditions

Following the re seizure of Mawdaung town by the Myanmar military on 19 May 2026, bunkers were observed at schools and station hospitals, indicating direct militarization of education infrastructure [727][730]. More broadly, the Myanmar military has been reported to target villages, schools, and religious buildings with airstrikes and drone attacks occurring almost daily in some areas, creating acute safety risks for school premises and staff [750]. A Civil Disobedience Movement teacher was reportedly executed during operations in Saw Township (Tapyin Village) in May 2026, illustrating direct threats to education personnel [749]. Data on violence against civilian services record 1,929 incidents against health care between 1 February 2021 and 12 May 2026, reflecting the sustained pattern of attacks that undermines civilian services including learning environments [730]. Intensified military operations and airstrikes have driven large-scale displacement — more than 5,000 people in Tanintharyi since mid-May 2026 and around 12,000 in western Bago since April — with humanitarian access constrained in these areas [734]. Displaced families are sheltering in forested and mountainous areas amid acute shortages of tarpaulins, shelter materials, and medical supplies, conditions under which schooling is severely disrupted [749].

Access to Education and Barriers

Intensified military operations since mid-May 2026 have driven new displacement that is directly constraining children's access to school, with more than 5,000 people displaced in Tanintharyi Region and around 12,000 remaining displaced in western Bago Region since April, and humanitarian access limited in both areas [734]. Overcrowding in temporary settlements and IDP camps, coupled with rising infectious disease risk and shortages of services and transport, is further limiting children's ability to attend school [726][742]. In southern Rakhine (Arakan), more than 3,000 IDPs across Taungup, Thandwe, and Gwa townships face severe shortages of assistance and livelihoods, and reports of displaced children increasingly begging point to disrupted schooling and serious child protection concerns [746]. Outside Myanmar, Rohingya refugees originating from Rakhine face persistent barriers to education and remain heavily reliant on aid, with UNHCR and partners warning that a \$710.5 million appeal that is only approximately 60% funded risks further reductions in education and other essential services for this population [729][731].

Education Needs and Current Response

Across conflict-affected areas of Myanmar, children require expanded access to safe learning spaces, trained teachers, adequate learning materials, and psychosocial support. Intensifying conflict and administrative obstacles — including NGO registration impediments and constrained partner access in many high-severity townships — are limiting humanitarian presence and the ability of organizations to deliver education and other services where they are most needed [726]. For Rohingya children from Myanmar in Bangladesh camps, UNHCR and partners report education gains since 2017 but warn that sharp funding cuts now threaten these services; the 2026 appeal for Rohingya refugees and host communities totals US\$710.5 million and was reported at roughly 60% funded, putting education, protection, and other services at risk [729][731]. WFP and other UN partners continue multisectoral assistance in Cox's Bazar and report coordinated adjustments to aid delivery, while noting that education services remain vulnerable to funding shortfalls and operational constraints [733][731].

Health

Availability and Access to Healthcare Services

Attacks on health care have been widespread and sustained, with 1,929 incidents of violence against health care recorded between 1 February 2021 and 12 May 2026; 66% attributed to the Myanmar Armed Forces; at least 173 health workers killed and 930 arrested; health facilities damaged on at least 503 occasions; at least 53 maternal health services affected; and more than 80 incidents involving drones [771]. Between 1 January and 31 May 2026, Insecurity Insight reported 73 incidents while WHO's SSA verified 38, with intensified airstrikes and drone use across Chin, Magway, Rakhine, Sagaing, Kachin, Karenni and northern Shan severely impeding access to care and medical supply transport [768]. An LINGO ambulance struck a landmine on 19 May 2026, killing one LINGO worker and injuring other staff and patients, illustrating the high security risks for health actors and patients in transit [770].

Health partners operated in only 106 of 227 prioritized townships (47%) during Q1 2026 and submitted interventions from just 30 of 86 townships classified as severity level 4 (35%), with limited access to the highest-need areas largely because 78% of level-4 townships are outside junta control and intensified clashes are impeding partner access and reporting [769]. Operational constraints include NGO registration problems forcing some actors to cease activities, conflict blocking transport of medical supplies, and poor internet and data-management capacity affecting reporting [769].

Recent hostilities and blockades are further disrupting medical supply chains: a junta-imposed blockade since 14 May has cut transport of essential goods, food and medicine from Pakokku toward western Yaw townships, and an offensive along the Chin–Magway border displaced roughly 14,000 people in early June, leaving displaced populations with acute shortages of medical supplies [789]. In Karenni State IDP camps, access barriers are particularly acute — travel times to the nearest health centre are approximately two hours, mobile health teams visit some camps only once every three months, and more than 10,000 people across four camps have been affected by malaria alongside widespread respiratory and skin diseases and severe medicine shortages [772].

Disease Outbreaks

Since late May–early June 2026, Myanmar has reported localized outbreaks of acute watery diarrhoea (AWD), malaria, dengue, mumps and persistent skin infections concentrated in conflict-affected areas and overcrowded IDP sites, driven by poor WASH, displacement, limited diagnostics and constrained access to health services [769][768][772].

AWD outbreaks have been reported in Karen, Karenni, Mon, Sagaing and Southern Shan; while rapid detection and response contained some cases early, limited rapid diagnostic tests, low risk awareness and chronically poor-quality drinking water sustain recurrence risk [769][768]. Malaria surges attributed to lack of vector control and bednets were specifically noted in Chin, Kachin and Tanintharyi, with inadequate testing and treatment enabling rapid expansion [768]. IDP camps in Karenni State reported simultaneous disease burdens at the onset of the rainy season, including more than 10,000 people across four camps with increased malaria, widespread respiratory illnesses, rising mumps among children since mid-May, and persistent skin disease, with clinics under-resourced and health teams visiting infrequently [772]. Heavy rainfall forecasts for early June are elevating the risk of waterborne disease and flooding, increasing AWD/cholera risk during monsoon onset [791].

Preparedness and response actions include subnational AWD Action Plan updates, expanded prepositioning of medical supplies by the Health Cluster, and a UNICEF-supported AWD preparedness workshop in Kachin on 4–5 June 2026, alongside intensified measles preparedness and health worker training in border areas at high risk from large outbreaks in neighbouring Bangladesh [769][768][791].

Health System Challenges

Attacks on health care have critically undermined health system functioning across Myanmar. Between 1 February 2021 and 12 May 2026, Insecurity Insight recorded 1,929 incidents of violence against health care, with 66% attributed to the Myanmar Armed Forces, at least 173 health workers killed, 930 arrested, and health facilities damaged on at least 503 occasions [770][771]. Intensifying airstrikes and drone use across Chin, Magway, Rakhine, Sagaing, Kachin, Karenni and northern Shan are severely impeding both access to care and the transport of medical supplies [768].

Conflict-related access restrictions and blockades have further disrupted medical supply chains, with junta checkpoints banning transport of medicines from Pakokku on 13 May 2026 and a subsequent blockade since 14 May cutting off essential goods to western Yaw/Mindat, leaving displaced populations with acute medicine shortages and limited healthcare access [770][789]. Health service coverage remains critically low: in Q1 2026, health partners provided services in 106 of 227 prioritized townships (47%) and submitted interventions from only 30 of 86 "extremely deprived" townships (35%), with 78% of high-severity townships located outside junta control [769].

Systemic humanitarian constraints compound these challenges, including NGO registration impediments with de facto authorities, funding and reporting difficulties, and limited partner access. The ongoing conflict has also driven disease escalation, with malaria surges in Chin, Kachin and Tanintharyi due to lack of vector control and bednets, and AWD outbreaks in Karen, Karenni, Mon, Sagaing and Southern Shan, while Karenni IDP camps report rising malaria, dengue, respiratory and skin diseases with medicines in very short supply [768][772].

Mental Health and Psychosocial Support (MHPSS)

The sustained campaign of violence against health care in Myanmar has severely undermined access to mental health and psychosocial services. The widespread attacks on health facilities and workers — including 930 health workers arrested and 503 occasions of facility damage since February 2021 — have reduced the overall health system capacity within which MHPSS services would ordinarily be delivered, with health partners reaching only 47% of prioritized townships as of Q1 2026 [770][771][769][768].

Community and civil society initiatives provide targeted psychosocial support in this constrained environment. The Assistance Association for Political Prisoners (AAPP) offers psychological support, online mental health training and psychological training for recently released political prisoners to reduce stigma and link them to professional services, while the ICRC assists by covering travel costs that help maintain prison–family contact and support mental well-being [786]. Digital repression, internet shutdowns and surveillance are reported to cause widespread anxiety and self-censorship, increasing psychosocial stress among civilians and raising protection risks for women's rights defenders and other activists [795]. UN reporting and atrocity monitoring note a sharp rise in conflict-related sexual violence in 2025, documenting its use as a tactic of political violence, and indicating increased need for survivor-centred MHPSS and funded survivor assistance [809].

Health Needs and Current Response

The most urgent health needs in Myanmar are concentrated among conflict-affected and displaced populations, particularly at the onset of the 2026 rainy season. Communicable disease threats include malaria surges in Chin, Kachin and Tanintharyi, AWD/cholera outbreaks in Karen, Karenni, Mon, Sagaing and Southern Shan, and rising skin infections, dengue risk and mumps among IDPs in Karenni [768][769][772]. In IDP settings, coverage is particularly weak: Karenni camps report clinic visits only once every three months, frequent medicine stockouts, and malaria outbreaks affecting tens of thousands across multiple camps, with roughly eight in ten clinic attendees showing malaria symptoms in some locations [772].

Critical gaps include shortages of diagnostics and medicines, lack of vector control and bednets, inadequate WASH driving recurrent AWD, insufficient prepositioned medical supplies for hard-to-reach areas, and partner constraints from NGO registration hurdles, internet and data limitations, and intensified clashes restricting access and reporting [769][768][772][789]. Sexual and reproductive health services are being delivered primarily through mobile and community-based modalities, with UNFPA releasing 29 Inter-Agency Reproductive Health kits to support CEmONC services [769].

Ongoing and planned responses include the Health Cluster's expansion of prepositioning of medical supplies to strategic hard-to-reach locations, initiation of Phase II of the Health Facility Survey, joint Health–Nutrition–WASH AWD action planning, a UNICEF-supported AWD preparedness workshop in Kachin on 4–5 June 2026, intensified measles preparedness in Rakhine, a UNFPA Pharmacist Working Group market assessment for medical suppliers, and upcoming NCD and localization workshops in June 2026 to strengthen partner preparedness and risk mitigation [769].

Nutrition

Availability and Access to Nutrition Services

Health partners reported providing services in 106 of 227 townships (47%) prioritized in the 2026 Humanitarian Needs and Response Plan, while only 30 of 86 townships categorized as 'extremely deprived' submitted data on partner interventions (35%) [813]. Key and sustained barriers to nutrition service delivery include widespread attacks on health care and access constraints: Insecurity Insight recorded more than 1,929 incidents of violence against health care between 1 February 2021 and 12 May 2026, with 66% attributed to the Myanmar Armed Forces, at least 173 health workers killed and 930 arrested, health facilities damaged on at least 503 occasions, and at least 53 maternal health services damaged, disrupted, or otherwise affected by violence, including more than 80 recorded drone-related incidents affecting health workers, facilities, transport, and service provision [816][819]. Intensified airstrikes and drone attacks in Chin, Magway, Rakhine, and Sagaing are impeding access to health care and the transport of medical and nutrition supplies, while junta checkpoints have at times banned the movement of medicines and food — further restricting supply chains [811][816]. Administrative constraints — including NGO registration issues imposed by de facto authorities — and escalating fighting are forcing some partners to cease activities and limiting access to high-severity townships [813].

Displaced populations face acute food and nutrition gaps: more than 3,000 IDPs in Taungup, Thandwe, and Gwa townships (southern Rakhine) face severe food shortages and minimal humanitarian support, with children increasingly resorting to begging for food [814], and renewed conflict in Arakan since November 2023 has contributed to roughly 600,000 people displaced and to prolonged blockades that undermine supply chains and livelihoods [814]. In eastern and border IDP sites, nutrition and related health services are extremely limited: several Karenni camps lack on-site clinics, health teams visit only once every three months, medicines and nutrition support are in very short supply, and malaria outbreaks have affected over 10,000 people across four camps with around eight in ten clinic patients showing malaria symptoms — all of which reduce the capacity for malnutrition prevention and treatment [818].

Joint Health–Nutrition–WASH preparedness actions are ongoing, including AWD risk indexing, AWD Action Plans, and prepositioning of medical supplies for rapid outbreak response; UNICEF supported AWD preparedness training in Kachin on 4–5 June 2026, indicating some coordinated preparedness efforts despite access constraints [811][813]. UNFPA released 29 Inter-Agency Reproductive Health (IARH) kits to SRH partners to support service delivery, including referral hospitals providing Comprehensive Emergency Obstetric and Newborn Care (CEmONC) services [813].

Nutrition Needs and Current Response

Health and nutrition actors report active preparedness and targeted nutrition delivery amid severe access constraints and disease outbreaks, but coverage gaps remain large [811][813]. WFP delivered nutrition services in April 2026 reaching 95,500 children (6–59 months) and 43,700 pregnant and breastfeeding women, and enrolled 4,710 children with moderate acute malnutrition in treatment programmes for Rohingya refugees [812]. Humanitarian coordination has prioritized joint Health–Nutrition–WASH AWD preparedness — including an AWD risk index, sub-national action plans, measles preparedness in Rakhine and Chin, health worker training, and expanded prepositioning of medical supplies for rapid outbreak response [811][813].

Intensified airstrikes, drone attacks, and other insecurity continue to impede access for health and nutrition teams and the transport of supplies, reducing treatment and prevention coverage [811].

Vector control gaps, lack of bednets, and shortages of medicines are driving malaria surges reported in Chin, Kachin, and Tanintharyi, complicating nutrition-sensitive disease prevention in IDP settings [811][818]. Beyond southern Rakhine, recent displacements in Tanintharyi (5,000+ since mid-May) and western Bago (approximately 12,000 since April) have been accompanied by reported shortages of food and basic supplies [815]. Funding shortfalls are also affecting refugee responses: UNHCR and partners note that appeals for Rohingya assistance are only partially funded, risking the sustainability of essential services including nutrition for vulnerable groups [824].

WASH

Availability of Water Sources

AWD outbreaks were detected in high-risk areas—Karen, Karenni, Mon, Sagaing, and Southern Shan—driven by chronic lack of hygiene and poor-quality drinking water, though a late monsoon onset kept case numbers lower than in 2025 [843][844]. Overcrowding in temporary and IDP settlements is a persistent driver of rapid AWD and skin disease spread as the rainy season begins, with limited WASH services and medical supplies in many camps [843][850]. Forecasting products flagged heavy rainfall and flood risk in early June 2026—including up to ~300 mm in 10 days and potential 1-in-20-year flooding affecting Yangon and Bago—heightening the risk of water-supply disruptions and waterborne disease spread during the monsoon [867]. UN agencies warned that blocked humanitarian access and funding shortfalls are leaving communities, including Rohingya refugees and newly displaced people, without reliable access to clean water and basic WASH services [868][860].

Access to Sanitation Facilities and Hygiene Practices

IDP camps in Karenni State report poor WASH conditions, medicine shortages, lack of nearby clinics, and urgent needs for mosquito nets as malaria and dengue surge alongside skin and respiratory infections [850]. Lack of vector control and insufficient bednet coverage are driving malaria increases in Chin, Kachin, and Tanintharyi, exacerbating WASH-related health risks [844]. Joint Health–Nutrition–WASH AWD action planning is underway at sub-national level as part of monsoon preparedness efforts, with limited rapid diagnostics and community awareness identified as key barriers to outbreak control [844][843]. Mass displacement around Mindat amid monsoon rains has left displaced populations exposed, with acute shortages of shelter, tarpaulins, food, and medical supplies, heightening hygiene and disease-transmission risks in affected sites [875].

Waste Management and Vector-Borne Disease Risks

Intensified conflict and access constraints are significantly reducing health partner presence and reporting in high-severity townships, limiting delivery of WASH and disease-prevention interventions [843]. Health partners active in only 106 of 227 prioritized townships (47%) and reporting interventions from only 30 of 86 severity level-4 townships indicate major coverage gaps that also affect waste management and vector control services [843]. A June risk bulletin warned of heavy mid-June rainfall and flooding risk, with Yangon and Bago states particularly exposed—conditions that could worsen water contamination and increase vector proliferation risks [867]. The Health Cluster identified chronic poor hygiene and unsafe drinking water, limited rapid diagnostic tests, and low community awareness as root causes enabling recurrent AWD outbreaks, and is updating sub-national AWD Readiness Action Plans alongside a multisectoral Health–WASH–Nutrition AWD Risk Index [843].

Main WASH Needs and Humanitarian Response

Key WASH needs identified include chronic lack of hygiene, poor-quality drinking water, inadequate sanitation in overcrowded temporary settlements, and limited rapid diagnostic testing and community awareness for AWD prevention [843]. Vector-control gaps and shortages of bednets are driving malaria surges in Chin, Kachin, and Tanintharyi, increasing WASH-adjacent needs in IDP and camp settings [844][850]. WASH Cluster preparedness and response measures include a Township Prioritization Tool for joint Health–Nutrition–WASH planning, updates to AWD Action Plans for Northwest, Karenni and Shan, Rakhine, and Southeast, and an AWD preparedness workshop in Kachin (Myitkyina) on 4–5 June 2026 organized by UNICEF [843]. Health and cluster partners are expanding prepositioning of medical and outbreak-response supplies to strategic, hard-to-reach locations to enable rapid deployment during AWD events [843]. Major constraints to scaling the WASH response include intensified armed clashes, airstrikes and drone attacks restricting access, NGO registration barriers, transport bans at checkpoints, and persistent connectivity and reporting limitations among partners [843][844][859].

Shelter and Non-Food Items (NFI)

Damage to Housing and Shelter Needs

Intense military operations and large explosions have caused widespread housing destruction and acute shelter needs across multiple states in early June 2026 [898][926][912]. In Mindat (Chin–Magway border), roughly 7,000 residents evacuated Mindat town and a further ~7,000 from 13 neighbouring villages since operations began on 13 May, joining some 10,000 long-term IDPs — putting Mindat township displacement at an estimated ~17,000 people; displaced families are sheltering in forests and mountains and face urgent shortages of tarpaulins, shelter, food and medical supplies ahead of the monsoon [898]. In Karenni State (May 2026), more than 120 civilian properties were recorded damaged, including 96 houses (with 85 houses and 16 vehicles destroyed by fire), plus damage to a school, a religious building and public-use facilities [903]. In Hpruso Township, over 80 homes were reported burned across three villages between March and May (16 houses on 28 March; >60 houses on 6 May; 4 houses on 7 May), with displaced civilians identified as needing shelter and food assistance [935]. In Northern Shan (Namhkam blast, 31 May), a massive explosion levelled much of a village — reports describe more than 100 houses near the blast damaged, with accounts that "over half" of the village's houses were destroyed [926][912][932].

In southern Rakhine/Arakan, some 3,000+ IDPs in Taungup, Thandwe and Gwa townships are sheltering in forested or mountain areas or makeshift camps and lack basic roofing and tarpaulins, leaving them exposed ahead of the monsoon [897]. Rebuilding and shelter response are further constrained by high repair costs and scarce construction labour and materials — for example, Inle Lake earthquake-damaged homes from 28 March 2025 remain unrepaired due to rising wages and material prices — as well as deliberate destruction of communal infrastructure such as schools and public buildings in conflict-affected areas, compounding obstacles to recovery and safe return [899][903].

Living Conditions in Camps or Settlements

Displaced populations across multiple regions face severe living conditions. Intensified operations since mid-May in Tanintharyi displaced more than 5,000 people sheltering in forested areas with severe shortages of food and basic supplies, while western Bago has around 12,000 people displaced since April with constrained humanitarian access and high needs for food, shelter and healthcare [906]. Overcrowding in temporary settlements is driving disease spread: Karenni State camps report surges in malaria, dengue, respiratory and skin infections with mumps rising among children since the second week of May, over 10,000 people across four camps in eastern Loikaw Township affected by malaria, and some camps (e.g., Kyaytone) sheltering more than 200 households (2,000+ people) with no clinic inside and health teams visiting only quarterly [902]. The Interim Executive Council of Karenni State reported 92,577 patients visited clinics it operates, and health workers expect patient numbers to rise with the rainy season [902]. The Health Cluster warned that overcrowding in temporary settlements heightens skin-disease transmission, that lack of vector control and bednets is triggering malaria surges in Chin, Kachin and Tanintharyi, and that recurrent AWD/cholera outbreaks are linked to poor hygiene and unsafe drinking water; health partners reported service delivery in 106 of 227 prioritized townships (47%) and in 30 of 86 townships classified as 'extremely deprived' (35%), reflecting limited coverage in high-severity areas [904][901]. Attacks on health care and access constraints are further undermining responses: Insecurity Insight recorded more than 1,929 incidents of violence against health care between 1 February 2021 and 12 May 2026 (66% attributed to the Myanmar Armed Forces), with at least 173 health workers killed, 930 arrested and at least 503 health facilities damaged; recent reported incidents include bans on transporting medicines from Pakokku and ambulances damaged by explosives [914][908]. Outside Myanmar, about 1.2 million Rohingya refugees — mostly in Cox's Bazar — remain heavily reliant on aid, with some 150,000 new arrivals since early 2024; UNHCR and partners launched a \$710.5 million 2026 appeal (hyper-prioritized) that is around 60% funded, and agencies warn that funding shortfalls, overcrowding and extreme-weather risk are undermining food, water, health and protection services in the camps [921][900][905][909].

NFI and Shelter Needs and Current Response

IDP and displaced communities report urgent needs for shelter materials (tarpaulins and basic roofing), safe temporary shelter, mosquito nets, medicines and other household NFIs ahead of the monsoon season [897][898][902][935]. Field reporting from Chin-Magway and Karenni areas highlights acute gaps in immediate NFI provision — including tarpaulins, shelter kits, mosquito nets and medicines — with frequent supply shortages and limited distribution reach leaving displaced families exposed to rain, mouldy rations and deteriorating living conditions [898][935][902]. Local aid workers and camp-management sources report that relief support is minimal to almost nonexistent in many areas of southern Rakhine and newly displaced sites, leaving thousands unable to procure basic shelter materials [897][898][935]. Access and logistics represent major constraints: deliberate blockades and transport bans — including interdictions on medicines and food from Pakokku since mid-May — and restrictions imposed by de facto authorities impede delivery of NFIs and have forced some partners to suspend activities [898][914]. Cluster-level preparedness actions are underway: the WASH Cluster developed a Township Prioritization Tool and UNICEF is convening AWD preparedness training (Kachin, 4–5 June 2026), while the Health Cluster is expanding prepositioning of medical supplies for rapid deployment to hard-to-reach areas [904]. Funding shortfalls continue to constrain the overall response, with the Rohingya appeal substantially underfunded at around 60% of its reduced target, pressuring the ability to sustain shelter and basic services for refugees and adding strain to regional humanitarian resources [900][921].

Protection

Impact on civilians

A major junta offensive and blockade along the Chin-Magway border triggered mass displacement in early June 2026, forcing roughly 14,000 people into surrounding forests and prompting about 7,000 evacuations from Mindat town plus another 7,000 from nearby villages; Mindat's total displaced population, including long-term IDPs, was reported at about 17,000 as of 2–3 June 2026 [964][947]. Displaced families face acute shortages of shelter, food, medical supplies and exposure to monsoon rains, with camp managers reporting moldy rations, a lack of tarpaulins and limited medical assistance for those hiding in the mountains [964]. Junta-imposed access restrictions and blockades have stopped transport of essential goods from Pakokku since mid-May, severely constraining deliveries of food and medicines to western Yaw region townships [964][955].

Violence against health care remains widespread, with Insecurity Insight recording more than 1,929 incidents across Myanmar between 1 February 2021 and 12 May 2026 — approximately 66% attributed to the Myanmar Armed Forces — including at least 173 health workers killed, 930 arrested, and health facilities damaged on at least 503 occasions; 73 attacks on health care were recorded between 1 January and 31 May 2026 [955][954][949][994]. The Myanmar Health Cluster reported intensifying airstrikes and drone attacks in Chin, Magway, Rakhine and Sagaing that are impeding access to services and medical supply transport [954][949]. Recent reported incidents include a 19 May landmine strike that damaged an LNGO ambulance, killing an LNGO worker and injuring patients and staff, and 24 May artillery shelling that damaged two buildings of a functioning station hospital in Natogyi township, Mandalay region [955].

Large-scale destruction of civilian property and forced displacement continued in May, with the Karenni Human Rights Group documenting over 120 damaged civilian houses, vehicles and public buildings in Karenni State in May 2026 and reporting around 7,000 people displaced from Hpruso Township following arson and military operations [945].

Protection risks have escalated, including reported executions and arbitrary arrests — six civilians executed in Tapyin village during May — and a patterned use of sexual violence as a tool of political repression, as noted in UN reporting on conflict-related sexual violence in Myanmar [964][969].

Rohingya populations remain heavily dependent on humanitarian aid and at heightened risk from funding shortfalls; UNHCR and partners launched a \$710.5 million appeal in late May/early June and warned the response was only around 60% funded, jeopardizing food, shelter, health and protection services for refugees linked to the Myanmar crisis [956][944]. Health partners report major coverage gaps: as of Q1 2026 they were providing services in 106 of 227 prioritized townships (47%), while many high-severity (level 4) townships — largely outside junta control — remain inaccessible to partners, limiting essential service delivery [954].

Factors Driving protection risks

Ongoing military operations, intensified airstrikes, drone strikes and widespread use of heavy weapons have escalated civilian harm and severely impeded access to health care and medical logistics across multiple states, including Chin, Magway, Rakhine, Sagaing, Kachin, Karenni and northern Shan [949][954][955]. Systematic attacks on health care are a persistent driver of protection risk, with armed-group activity, landmines and unsafe explosive storage also producing direct civilian casualties and disrupting aid operations — highlighted by cross-border landmine and mortar casualties near Bangladesh in late May–early June 2026, and a large accidental explosives blast in Namhkam at end-May 2026 linked to stored mining explosives [955][980][966][961][960].

Arbitrary arrests, detention and intensified digital repression — including revocation of publishing licences on 11 May 2026, documented arrests for online expression, and reported deaths of political prisoners in late May 2026 — are shrinking civic space and producing detention-related protection harms [950][943]. Targeting of livelihoods (including reported attacks on rice fields and widespread village burnings), massive internal displacement and reduced humanitarian funding are driving acute economic deprivation and dependency, with refugee and displaced populations including Rohingya facing heightened vulnerability and about 150,000 new arrivals from Rakhine since early 2024 relying on aid [950][951][944][948].

Legal and institutional weaknesses and information blackouts are erecting barriers to protection and justice, while constrained humanitarian access is illustrated by health partners reaching services in only 47% of the 227 prioritized townships and reporting interventions in just 35% of townships categorised as extremely deprived for health [954][950][943]. These intersecting drivers are producing mounting civilian casualties, repeated displacement, restricted access to essential services and erosion of basic protections, with recent localized displacements — including more than 5,000 people in Tanintharyi since mid-May 2026 and roughly 12,000 displaced in western Bago since April 2026 — underscoring the scale of need [942][941][949].

Psychological and mental health impacts

The Assistance Association for Political Prisoners (AAPP) provides psychological support and online mental-health/self-rehabilitation training for former political prisoners experiencing PTSD, depression and anxiety, and delivers training intended to reduce mental-health misconceptions and link released prisoners with professional services [965]. AAPP figures indicate the regime continues to hold more than 22,000 political prisoners while nearly 9,000 have been released, indicating a large population with potential MHPSS needs [965]. While civil society groups and ICRC assistance (e.g., covering travel costs to prisons) provide some support to detainees and families, reporting highlights persistent gaps in comprehensive MHPSS coverage, access to professionals and targeted services for vulnerable groups including former prisoners, displaced populations and survivors of violence [965].

Violence against health care has degraded access to mental-health services, and WHO's Surveillance System for Attacks on Health Care recorded 38 verified attacks between 1 January and 31 May 2026; Health Cluster analysis shows "psychological violence" featured in 11% of reported incidents in early 2026, signalling direct threats to both provider and patient mental well-being and added barriers to safe care [955][954][994]. The Health Cluster has planned SSA awareness sessions to encourage incident reporting by partners, reflecting recognition of reporting and monitoring gaps that affect protection and MHPSS planning [954].

Intensifying conflict and displacement — including the roughly 14,000 people forced into mountains and camps along the Chin–Magway border in mid-May–early June 2026 — have left displaced populations facing acute shortages of shelter, food and medicine in monsoon conditions, exacerbating trauma and psychosocial distress [964]. Local incident reporting further documents injured families facing high treatment costs, displacement and no external support, illustrating economic barriers to recovery and gaps in psychosocial assistance for survivors of attacks and shelling [959][982].

Protection Needs and Current Response

Protection needs across Myanmar remain acute, with escalating violence, constrained partner presence and access impediments leaving high-severity areas and displaced populations severely under-served. Key figures as of Q1/mid-2026 include:

- Health partners providing services in 106 of 227 prioritized townships (47%) as of Q1 2026 [954].
- Of 86 townships classified as severity level 4 for health, partners submitted interventions from only 30 townships (35%) — many high-severity areas remain inaccessible [954].
- More than 5,000 people displaced in Tanintharyi since mid-May 2026 and around 12,000 in western Bago since April, many sheltering in forests or makeshift sites and facing acute shortages of food and basic supplies [942][952].
- More than 3,000 IDPs in southern Rakhine (Taungup, Thandwe, Gwa) face severe food insecurity, minimal humanitarian access and inadequate shelter ahead of the monsoon season [952].

On the humanitarian response side, the Health Cluster is scaling reporting and monitoring through SSA awareness sessions to incentivize partners to report attacks, and expanding prepositioning of medical supplies for rapid deployment to hard-to-reach areas [954].

UNICEF is supporting AWD/cholera preparedness training in Kachin on 4–5 June 2026, and UNFPA released 29 Inter-Agency Reproductive Health kits to support SRH services [954]. However, registration obstacles imposed by de facto authorities are forcing some NGOs to cease activities, and checkpoints and movement restrictions — including bans on transporting medicines from Pakokku to villages in Magway Region — are further constraining protection and assistance delivery [954][994]. The killing of an LNGO worker and damage to an ambulance on 19 May 2026 continue to underscore risks to frontline responders and patients [955][994]. Overall, persistent gaps in protection monitoring, case management and safe shelter, combined with partner coverage under 50% of prioritized townships, leave the most vulnerable populations critically exposed [954][952].

Child Protection concerns

A large explosion in Namhkam, northern Shan State (late May–early June 2026) killed dozens including children and injured many, creating urgent protection and displacement needs for affected children and families [961][966][988][960]. Cross-border spillover violence has directly harmed children near the Bangladesh border, including the reported killing of a child by an abandoned mortar on 2 June 2026 [980]. Severe food shortages among more than 3,000 IDPs in southern Rakhine (Taungup, Thandwe, Gwa) have driven children to beg for food, signalling rising child protection and child-labour risks [952].

UNHCR and partners warn that about 1.2 million Rohingya refugees in Bangladesh face reduced assistance, limited access to education and heightened protection risks, with nearly 900 Rohingya reported missing or dead at sea in 2025 — factors that increase vulnerabilities for children and adolescents [944][963][948][956]. Intensified displacement inside Myanmar — including more than 5,000 people in Tanintharyi since mid-May 2026 and around 12,000 in western Bago since April — is exposing children to interrupted services and protection threats [942]. The presence of armed groups and indicators of recruitment and criminal activity around camps and borderlands raise risks of recruitment, exploitation and gender-based abuses affecting children and young people [980][969].

Disruptions to health and education services — including large gaps in partner coverage of high-severity townships and measles preparedness concerns in border areas with many previously unvaccinated children — further undermine child protection by reducing access to vaccination, learning and child-friendly services [954][949].

Violence against women and girls

Sexual violence is being used as a tactic of political violence in Myanmar, contributing to patterns of conflict-related sexual violence documented by the UN Secretary-General's report and subsequent monitoring analyses [969]. The Burmese Women's Union documented 44 women killed in April amid junta air and ground attacks, indicating heightened protection risks for women and girls, while arrests of women — 114 women among 374 people detained in a recent MIP tally — and digital repression and communications blackouts are increasing barriers to reporting GBV and to women's ability to reach services [943][950]. At least 53 maternal health services have been reported damaged or disrupted as part of the broader pattern of attacks on health care [955][994][949][954].

UNFPA and health partners have maintained GBV/SRH response capacity where possible: UNFPA released 29 Inter-Agency Reproductive Health (IARH) kits to partners and organized two Clinical Management of Rape (CMR) capacity-building sessions (one virtual, one in-person) around 1 June 2026, while SRH delivery has relied largely on mobile and community-based approaches to reach underserved populations [954]. Funding cuts and blocked humanitarian access continue to weaken prevention, monitoring and survivor assistance; UNHCR warned in early June 2026 that women and girls among Rohingya refugees are particularly affected by reduced aid and service shortfalls [956][948].

Vulnerable Groups / Special Needs Groups

Unaccompanied and Separated Children

No data available for this section.

Pregnant and Lactating Women

Attacks on health care have been extensive since the 2021 coup, with 1,929 incidents recorded up to 12 May 2026 and at least 53 maternal health services reported damaged, disrupted, or otherwise affected, reducing access to care for pregnant women, mothers and newborns [1009][1007]. Insecurity Insight reported 73 incidents affecting health care between 1 January and 31 May 2026, while WHO's SSA recorded 38 verified attacks in the same period, highlighting an ongoing surge in hostile incidents that undermine maternal health service delivery [1006]. Intensified airstrikes and drone use across Chin, Magway, Rakhine, Sagaing, Kachin, Karenni and northern Shan have severely impeded access to health care and the transport of medical supplies needed for maternal services [1006]. Conflict-related restrictions on movement and blockades have further disrupted medical supply chains — for example, junta checkpoints banned transport of medicines and food from Pakokku on 13 May 2026, and two buildings of a station hospital were damaged by artillery on 24 May 2026 — further reducing access to maternal care [1009].

Health partners reported providing services in 106 of 227 Humanitarian Needs and Response Plan priority townships (47%) during Q1 2026, and interventions were reported from only 30 of 86 townships categorized as "extremely deprived" (35%), indicating large gaps in coverage where pregnant and breastfeeding women are likely to be underserved [1013]. Sexual and reproductive health (SRH) services have mainly been delivered through mobile and community-based approaches to reach underserved populations, and UNFPA released 29 Inter-Agency Reproductive Health (IARH) kits to SRH partners, including supplies for referral hospitals providing Comprehensive Emergency Obstetric and Newborn Care (CEmONC) [1013]. Internally displaced and remote populations report critical gaps relevant to maternal health: clinics absent or visited infrequently, long travel times to the nearest health centre, shortages of medicines, and rising malaria and other infectious disease risks that compound vulnerabilities for pregnant women and infants in IDP camps [1015].

In the Rohingya refugee response in Bangladesh, WFP reported delivering nutrition services in April 2026 that reached 43,700 pregnant and breastfeeding women, underscoring high nutrition needs among displaced pregnant and lactating populations [1010]. Patterns of conflict-related sexual violence and worsening violations of women's rights have been documented in Myanmar, with monitoring groups reporting increases in violence against women and women killed in attacks, creating protection risks that intersect with maternal health needs [1055][1019]. Operational constraints — including NGO registration barriers, intensified conflict limiting access, and connectivity and reporting challenges — continue to impede partners' ability to deliver and monitor maternal health services, prompting Health Cluster plans for SSA awareness sessions, expanded prepositioning of medical supplies, and further facility and market assessments to support service continuity [1013].

Adolescents and Youth

Airstrikes, drone strikes and shelling have regularly targeted villages and schools across Myanmar, driving displacement and interrupting education for children and adolescents [1019][1006]. Intensified operations since mid-May 2026 displaced more than 5,000 people in Tanintharyi and left around 12,000 displaced in western Bago since April, with constrained humanitarian access and elevated needs that undermine schooling and youth livelihoods [1014]. In southern Rakhine/Arakan State, over 3,000 IDPs in Taungup, Thandwe and Gwa face acute food insecurity and minimal relief, and aid workers report an increase in displaced children begging as households lack income or assistance [1012].

Rohingya refugees and around 150,000 new arrivals from Rakhine face limited education and economic opportunities in camps; UNHCR warned funding cuts are threatening essential services and prospects for return are fading, while WFP and partners note continued progress in refugee education but persistent funding shortfalls that affect household stability [1008][1010]. Reports describe growing militarisation of the Cox's Bazar camps and surrounding areas, with armed recruitment and criminal economies identified as risks inside and around the camps [1022]. Psychosocial support is provided by groups such as the Assistance Association for Political Prisoners (AAPP), which offers psychological support and training to former political prisoners, but widespread attacks on health care and restricted partner access are eroding the availability of mental-health and other services for affected youth populations [1023][1009][1013].

People with Disabilities

According to UNHCR, persons with disabilities among the roughly 1.2 million Rohingya refugees in Bangladesh are among the most vulnerable and are disproportionately affected by sharp cuts in humanitarian and development assistance [1005][1008][1004]. In May 2026, the UN and the Government of Bangladesh launched an appeal for \$710.5 million to meet urgent needs for Rohingya refugees and host communities, but the appeal was only about 60% funded and 26% lower than last year, increasing the risk of reduced healthcare, education and protection services that persons with disabilities rely on [1008][1004]. UNHCR cautioned that maintaining only minimal levels of assistance could lead to more complex and costly humanitarian consequences for the most vulnerable groups, explicitly naming persons with disabilities among those at heightened risk [1005][1004].

Elderly

In Karenni State, elderly IDP residents with chronic conditions such as hypertension and diabetes are unable to afford medicines and face extremely limited access to clinics and mobile health teams, with outbreaks and medicine shortages reported at the onset of the rainy season in May 2026 [1015]. The Health Cluster (1 June 2026) reported that intensified airstrikes and drone attacks are severely impeding access to health care and transport of medical supplies, and announced plans to expand prepositioning of medical supplies, conduct SSA awareness sessions, and deliver additional noncommunicable-disease-in-emergencies workshops to support NCD care for older people [1013][1006].

Blockades and checkpoint restrictions since mid-May — including bans on transporting medicines and food from Pakokku and a junta blockade cutting supplies to western Yaw region since 14 May — have left displaced communities, including older people, without medicines and basic supplies [1009][1021]. Displacement, reduced humanitarian presence and mobility constraints documented in late May–early June 2026 have increased protection risks and social isolation for older people across multiple regions and IDP sites [1014][1012][1004]. UNHCR and partners warn that funding cuts and a hyper-prioritised 2026 appeal (around 60% funded) are reducing services for Rohingya refugees and other displaced populations, with older people explicitly identified among the most affected vulnerable groups [1004][1005].

Displaced

Key displacement and refugee figures reported in early June 2026 include the following:

- More than 5,000 people displaced in Tanintharyi Region after intensified operations since mid-May 2026 [1014].
- Around 12,000 people remain displaced in western Bago Region since April 2026 amid continued airstrikes and insecurity [1014].
- Roughly 14,000 residents evacuated Mindat town and nearby villages since mid-May 2026, joining about 10,000 long-term IDPs for an estimated 17,000 people displaced in Mindat township [1021].
- Since renewed fighting in November 2023, about 600,000 people have been displaced across Arakan (Rakhine) State, with pockets — such as over 3,000 IDPs in southern Arakan townships — facing acute food insecurity and minimal humanitarian assistance [1012].
- Some 1.2 million Rohingya refugees remain in Bangladesh (mostly in Cox's Bazar), including approximately 150,000 new arrivals since early 2024; a 2026 UN-led appeal for their needs totals US\$710.5 million and was reported about 60% funded at the time of reporting [1008][1004][1005][1016][1026].

Displaced households are sheltering in forested and mountainous areas, with many living in temporary makeshift camps with inadequate shelter supplies (lack of tarpaulins and roofing), limited income sources and rising reliance on begging and irregular coping strategies ahead of the monsoon season [1012][1021]. Livelihood opportunities are scarce in displacement-affected areas; local economies have collapsed in many frontline areas and displaced families report virtually no casual labour or formal employment, increasing food insecurity and dependence on shrinking aid supplies [1012].

Health and basic services are severely constrained: health partners reported providing services in 106 of 227 prioritized townships (47%), and only 30 of 86 townships classed as severity level 4 have partner interventions (35%), reflecting major access gaps in conflict-affected areas [1013]. Overcrowded IDP sites are already seeing surges in infectious diseases (malaria, dengue, AWD, mumps and skin infections), driven by poor WASH, medicine shortages and limited mobility to reach clinics [1015][1013]. Attacks on health care and intensified use of heavy weapons, airstrikes and drones — with hundreds of documented incidents since 2021 and dozens reported in early 2026 — have damaged facilities, killed and arrested health workers, and further undermined service delivery and disease surveillance [1009][1006][1007].

WFP reported adapting targeting in the Bangladesh refugee operation and supported 1.13 million Rohingya in Cox's Bazar in April 2026 while continuing to face operational strains during program changes [1010]. Prospects for safe return remain limited: UNHCR and partners report that ongoing conflict, persecution and insecurity inside Myanmar continue to prevent voluntary, safe and dignified return for refugees, contributing to secondary movement and dangerous sea crossings — nearly 900 Rohingya were reported dead or missing in regional sea crossings in 2025 — as people seek alternatives [1008][1004][1005][1016][1026].

Protection risks for displaced populations include targeted military offensives, blockades that cut supply routes (notably transport blockages from Pakokku toward western Yaw region since 14 May 2026), intensified air and drone attacks, and shrinking humanitarian access that limits assistance and heightens exposure to violence and rights violations [1021][1006][1014]. Rohingya in Malaysia lack formal legal status and face barriers to legal employment, education and basic services, even as Malaysia in 2026 rolled out a refugee registration system that had registered some 4,010 refugees (many detained at immigration depots) for biometric profiling and potential labour matching [1037]. Funding shortfalls combined with access constraints and sustained conflict thus compound protection, health, shelter and livelihood gaps for both internally displaced people inside Myanmar and refugees from Myanmar in neighbouring countries [1008][1013][1012][1006].

Minorities

About 1.2 million Rohingya refugees remain in Bangladesh, with roughly 150,000 new arrivals since early 2024, continuing to rely heavily on humanitarian assistance; women and girls, people with disabilities and older people are disproportionately affected by the crisis [1004][1008]. The UN-led appeal to meet urgent needs for Rohingya refugees and host communities totalled about \$710.5 million for 2026 and was reported at roughly 60% funded — a funding request 26% lower than the previous year — raising concerns about cuts to essential services [1008][1010]. WFP introduced a food-insecurity-based Targeting and Prioritization Exercise effective 1 April 2026 that adjusts monthly entitlements by household food security while maintaining coverage for all households, and in April WFP reached 1.13 million refugees in Cox's Bazar and 28,000 in Bhasan Char with general food assistance [1010]. With prospects for safe return fading, many Rohingya resort to perilous sea journeys; 2025 was recorded as the deadliest year for such crossings, with nearly 900 reported dead or missing in the Andaman Sea and Bay of Bengal [1008].

Violence against health care is widespread and sustained: Insecurity Insight recorded 1,929 incidents affecting health care in Myanmar between 1 February 2021 and 12 May 2026, with 66% attributed to the Myanmar Armed Forces, at least 173 health workers killed, 930 arrested, and health facilities damaged on at least 503 occasions; the growing use of drones has further endangered services [1009]. In Rakhine (Arakan) State, renewed conflict since November 2023 has displaced around 600,000 people, and a junta-imposed blockade has disrupted transport and supply chains, while more than 3,000 IDPs in southern townships face acute food insecurity amid very limited aid presence [1012].

Sexual violence has been documented as part of the repertoire of political violence in Myanmar, used to intimidate, punish and target communities along shifting frontlines and on ethnic grounds [1055]. Digital repression and communications blackouts — alongside arrests targeting online expression — disproportionately endanger women, women's rights defenders and activists, and hamper communication with families in areas such as Rakhine, increasing exclusion and protection risks [1027]. International accountability measures relevant to Rohingya abuses include proceedings before the International Court of Justice and an Argentinian arrest warrant issued in 2025 against Myanmar's military leader Min Aung Hlaing for alleged crimes against the Rohingya [1034].

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